

Patient: **BARON BROWN**  
Medical Record: **#107145**

# INITIAL COMPREHENSIVE ASSESSMENT (WITH HOPE)

## COMPREHENSIVE ASSESSMENT

**Date/Time In:** 12/30/2025 5:00:00 PM  
**Date/Time Out:** 12/30/2025 6:00:00 PM  
**Modality:** In-person visit  
**Describe onset of illness, progression of symptoms, functional status, response to treatments, etc: including any recent hospitalizations, ER visits, weight loss and infections:** Patient: Baron Lloyd Brown  
DOB: 10/26/1958 (Age 67)  
Gender: Male  
Marital Status: Divorced  
Race: Black or African American  
Religion: Baptist  
Primary Language: English  
Level of Care: Routine Hospice Care  
Allergies: Penicillin  
Code Status: DNR not elected  
Disaster Code: 2

Primary Hospice Diagnosis:  
I25.10 – Atherosclerotic heart disease of native coronary artery without angina pectoris

Comorbid Conditions:  
Pain, edema, diarrhea, acute respiratory distress, anxiety disorder, major depressive disorder (recurrent), essential hypertension, nausea, vomiting, constipation, personal history of nicotine dependence, pneumonia due to Streptococcus pneumoniae, nutritional deficiency, and fluid overload.

Mr. Brown is a 67-year-old male admitted to hospice services with advanced atherosclerotic heart disease complicated by multiple comorbidities and evidence of ongoing functional and nutritional decline. He is currently appropriate for Routine Hospice Care based on disease progression, symptom burden, and decreased physiological reserve.  
The patient has a Palliative Performance Scale (PPS) score of 50%, reflecting reduced ambulation, inability to carry out normal work activities, and increasing dependence due to fatigue and cardiopulmonary limitations. He reports progressive shortness of breath, particularly with minimal exertion, which is consistent with advanced cardiac disease and is further exacerbated by a significant history of tobacco use and prior pneumonia, contributing to decreased cardiopulmonary reserve.

Mr. Brown demonstrates chronic bilateral lower extremity edema, consistent with fluid overload and impaired cardiac function. He also reports episodes of acute respiratory distress, requiring close symptom monitoring and comfort-focused management.

Over the past two months, the patient has experienced a 10-pound unintentional weight loss and is now consuming approximately 50% of meals, indicating nutritional decline and poor intake, which are consistent with advanced heart disease and systemic deconditioning. He carries a diagnosis of nutritional deficiency, further supporting disease-related decline.

The patient experiences persistent pain, gastrointestinal symptoms (nausea, vomiting, diarrhea, constipation), and psychological distress, including anxiety and recurrent major depression, all of which negatively impact quality of life and functional status. Despite ongoing medical management, symptoms remain burdensome and progressive.  
The patient meets hospice eligibility criteria as evidenced by:

- Progressive decline in functional status (PPS 50%)
  - Ongoing symptoms of dyspnea and fluid overload
  - Poor nutritional intake with significant recent weight loss
  - Multiple comorbid conditions contributing to overall clinical decline
  - No expectation of meaningful recovery or stabilization with continued disease progression
- The patient is no longer focused on disease-modifying or aggressive interventions and is appropriate for comfort-directed, palliative-focused care.

- Pain and dyspnea control
- Management of edema and gastrointestinal symptoms
- Psychosocial and emotional support
- Nutritional support and comfort-focused interventions
- Interdisciplinary team involvement to support quality of life
- Mr. Brown's prognosis is consistent with a life-limiting illness, and hospice care is medically necessary and appropriate at this time.

CO-MORBIDITIES

What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

- ☒ Congestive Heart Failure
- ☒ Ischemic Heart Disease
- ☒ Coronary Artery Disease
- ☒ Generalized Muscle Weakness

GENERAL INFORMATION

- Race: African-American
- Religion: Christian
- Primary Language: English
- Does the patient or primary caregiver need an interpreter? No
- Is the patient or the patients spouse a veteran? No

PSYCHOSOCIAL / SPIRITUAL / BEREAVEMENT ASSESSMENT

Patient/Responsible Party's Current Response to Diagnosis/Prognosis: (Check all that apply)

• Denial

- ☒ Patient
- ☒ Representative

• Anger

- ☒ Patient
- ☒ Representative

• Bargaining

- ☒ Patient
- ☒ Representative

• Acceptance

- ☒ Patient
- ☒ Representative

• Support Systems

- ☒ Church
- ☒ Neighbors

☒ Friends

• *Spiritual Support*

Needs spiritual support? Yes

• *Psychosocial Support*

Needs psychosocial support? Yes

• *Patient/Family Goals*

- ☒ Patient and family will receive physical, spiritual, and emotional support
- ☒ Effective grieving
- ☒ Safe and comfortable dying

PAIN/COMFORT ASSESSMENT

Has patient experienced pain the past 7 days? Yes

Pain Assessment Scale utilized: Verbal (scale of 1-10)

Acceptable level of pain: 0

Best: 1

Worst: 8

☒ Generalized pain/undefined location

Pain Frequency: Intermittent

Pain Duration: 1-4 hours

Bone/Somatic pain: Aching Dull

Visceral pain: Aching

Neuropathic pain: Burning

*Non-Verbal Signs of Pain*

- ☒ Moaning/crying
- ☒ Grimacing

*What makes the pain better?*

- ☒ Rest
- ☒ Heat
- ☒ Cold
- ☒ Breathing exercises
- ☒ Medication

*What causes pain or makes it worse?*

- ☒ Position change
- ☒ Not taking pain medication

*What other symptoms are associated with the pain?*

- ☒ Decreased socialization
- ☒ Decreased appetite
- ☒ Nausea/vomiting
- ☒ Depression

Current treatments for pain:

☒ Opioids

Patient is on a scheduled opioid, currently taking a PRN opioid or has been instructed to begin taking an opioid: Yes

Specify the scheduled opioids the patient is currently taking (leave blank if n/a): Morphine

Specify the PRN opioids the patient is taking (leave blank if n/a): Morphine

Patient is on a bowel regime: Yes

Date bowel regime initiated: 12/30/2025

Describe bowel regime: Senna

How much breakthrough pain medication has patient taken in the past 24 hours? 0

Is current pain regime effective? Yes

Patient/family pain management goals Relieve pain/discomfort as much as possible even if it will cause increased sedation

Patient/caregiver educated on pain regime and side effects of pain medications: Yes, and verbalized understanding

Patient educated on non-medication treatments of pain? Yes

☒ Heat

☒ Cold

☒ Breathing exercises

Non-medication treatments were initiated: Yes

☒ Heat

☒ Cold

☒ Breathing exercises

☒ Massage

HEAD TO TOE ASSESSMENT

Vital Signs

Type of temperature taken: Temporal:

Temporal temperature: 97.1

Respiratory rate (respirations per minute): 18

Respiratory pattern: Regular

SaO2 93

Pulse Type: Radial

Heart rate: 66

Blood pressure type: Sitting

Systolic: 140

Diastolic: 78

Height: 6'3"

Weight: 180

Arm circumference taken: Left

Arm circumference: 28

Nutrition

☒ Diet Orders (specify):

☒ General (as tolerated)

Average percentage of meals consumed: 50  
Patient Feeding: Patient able to feed self independently

Neurological Status

Orientation

☒ Self

Behavior

- ☒ Calm
- ☒ Withdrawn
- ☒ Confused
- ☒ Forgetful

Energy

- ☒ Drowsy
- ☒ Fatigue/Weakness
- ☒ Increased sleep

Number of hours spent sleeping in a day: 15

Responsiveness

Responsiveness: Verbal stimuli

Intellect

Intellect: Confusion

Speech

- ☒ Garbled
- ☒ Slurred

Neuromusculoskeletal Status

- ☒ Shuffling gait/uncoordinated gait

Muscle Tone

- ☒ Muscle wasting

Functional Assessment

Functional Assessment Score: 6A

Gastrointestinal Status

Last BM Today

Bowel Sounds positive in all 4 Quadrants

- ☒ Hypoactive
  - ☒ RUQ
  - ☒ RLQ
  - ☒ LUQ
  - ☒ LLQ

☒ Constipation

Genitourinary Status

☒ Urinary incontinence

Integumentary Status

☒ Skin intact

Pressure Ulcer Risk Assessment

|                                                                                     |   |
|-------------------------------------------------------------------------------------|---|
| Sensory perception: ability to respond meaningfully to pressure-related discomfort: | 1 |
| Moisture: degree to which skin is exposed to moisture:                              | 1 |
| Activity: degree of physical activity:                                              | 1 |
| Mobility: ability to change and control body position:                              | 1 |
| Nutrition: usual food intake pattern:                                               | 1 |
| Friction and shear:                                                                 | 1 |
| Total score:                                                                        | 6 |

CARDIOPULMONARY ASSESSMENT

Was patient screened for shortness of breath 1. Yes  
Date of first screening for shortness of breath 12/30/2025  
Did screening indicate the patient had shortness of breath 1. Yes  
Was treatment for shortness of breath initiated 2. Yes  
Date treatment for shortness of breath initiated 12/30/2025

Respiratory Treatments for Shortness of Breath:

☒ Other current respiratory medications - opioid  
Specify: morphine

☒ Oxygen  
Liters: 2  
Via: NC  
Frequency: As needed

☒ Dyspnea  
☒ At rest  
☒ With exertion

☒ Decreased  
☒ RLL  
☒ LLL

☒ Cough  
Frequency: Chronic  
Type: Dry

☒ Capillary Refill (Fingers)  
☒ Less than 3 seconds

☒ Edema  
Location: lower extremities

Type: Non-pitting  
NY Heart Association Scale: Class IV (Severe)

FUNCTIONAL STATUS

Walking/Locomotion Walker  
Bed Mobility; Assist of 1  
Transfers: Assist of 1  
Dressing: Independent  
Eating: Independent  
Toilet use: Dependent  
Personal hygiene: Dependent  
Bathing Dependent  
Palliative performance scale (PPS): 50% - Unable to do any Work

Fall Risk Assessment (NHPCO Guidelines)

- History of a fall within the last 6 months
    - ☒ 3: More than 2 falls
  - Mobility
    - ☒ 2: Has a gait and/or balance problem
  - Elimination
    - ☒ 2: Intermittent incontinence
  - Mental Status
    - ☒ 2: Intermittent confusion
  - Use of medication
    - ☒ 1: Narcotic
  - Medical conditions which could impair the ability to ambulate
    - ☒ 1: Weakness or paralysis of the extremities
- Total: 11  
☒ 10 or more points = High risk

HOME SAFETY ASSESSMENT

☒ No safety problems identified

ASSESSMENT OF PATIENT/CAREGIVER TO SAFELY ADMINISTER MEDICATIONS

- ☒ N/A (Medication administered by facility staff)
- ☒ Patient/Caregiver/POA verbalized understanding of Medication teaching provided

HOPE Admission Assessment

Based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less? 0. No  
Living Arrangements 2. With others in the home (family, friends,

Availability of Assistance

paid caregiver)  
1. Around-the-clock (24 hours a day with few exceptions)  
0. No

Interpreter requested

Preferences

Was patient/responsible party asked about CPR 1. Yes  
Date asked about CPR 12/30/2025  
Was patient/responsible party asked about treatments other than CPR 1. Yes  
Date asked about treatment other than CPR 12/30/2025  
Was patient/responsible party asked about hospitalization 1. Yes  
Date asked about hospitalization 12/30/2025  
Was patient/responsible party asked about spiritual/existential concerns 1. Yes  
Date asked about spiritual/existential concerns 12/30/2025

Active Diagnoses

Principal Diagnosis 05. Chronic Obstructive Pulmonary Disease (COPD)

Comorbidities and Co-existing Conditions

- ☒ Heart Failure (e.g., congestive heart failure (CHF) and pulmonary edema)
- ☒ Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- ☒ Renal disease

Pain

Was patient screened for pain 1. Yes  
Date of first screening for pain 12/30/2025  
Patient's pain severity was 2. Moderate  
Type of standardized pain tool used 1. Numeric  
Is pain an active problem for the patient? 1. Yes  
Was comprehensive pain assessment done 1. Yes  
Date of comprehensive pain assessment 12/30/2025

Comprehensive pain assessment included:

- ☒ Location
- ☒ Severity
- ☒ Character
- ☒ Frequency
- ☒ Duration
- ☒ What relieves/worsens pain
- ☒ Effect on function or quality of life

Neuropathic Pain 1. Yes

Symptom Impact

Was a symptom impact screening completed? 1. Yes  
Date of symptom impact screening 12/30/2025  
Pain 1. Slight  
Shortness of breath 1. Slight  
Anxiety 1. Slight  
Nausea 0. Not at all



|              |               |
|--------------|---------------|
| Vomiting     | 0. Not at all |
| Diarrhea     | 0. Not at all |
| Constipation | 1. Slight     |
| Agitation    | 0. Not at all |

**Skin Conditions**

Does the patient have one or more skin conditions? 0. No

**Medications**

Was scheduled opioid initiated or continued? 0. No  
Was PRN opioid initiated or continued? 1. Yes  
Date PRN opioid initiated or continued 12/30/2025  
Was bowel regimen initiated or continued? 2. Yes  
Date bowel regimen initiated or continued 12/30/2025

**TEACHING/EDUCATION PROVIDED**

**Education Provided To:**

- ☒ Patient
- ☒ Caregiver

**Topics**

- ☒ Palliative care, hospice philosophy, hospice benefit
- ☒ Scope of services, role of team members
- ☒ Call us First Program
- ☒ Diagnosis
- ☒ Prognosis
- ☒ Disease process
- ☒ Safe & effective use of equipment & supplies
- ☒ Dying process, signs & symptoms of approaching death
- ☒ Covered medications, supplies, equipment
- ☒ How to re-order medications and supplies
- ☒ Medication usage, side effects, drug interactions
- ☒ Basic home safety, emergency preparedness
  - ☒ Provided written personalized emergency preparedness plan
  - ☒ Resides in a facility. Follow facility plan
- ☒ Infection control, disposal of hazardous waste, universal precautions
- ☒ Skin care, wound care, dressing changes
- ☒ Energy Conservation techniques
- ☒ Plan of care established with primary care giver
- ☒ Symptom management
- ☒ Catheter care
- ☒ Bowel regimen

Pain management

- ☒
- ☒ What to do at the time of death
- ☒ Anticipatory grief, grief, bereavement
- ☒ Funeral planning
- ☒ Advance directives
- ☒ Community resources
- ☒ Signs & symptoms of infection
- ☒ Discharge planning

**COMMUNICATION WITH OTHER TEAM MEMBERS**

- ☒ Hospice Medical Director
- ☒ Clinical Director/PCC
- ☒ Assigned RN Case Manager
- ☒ Assigned LPN
- ☒ Social Worker
- ☒ Volunteer Coordinator
- ☒ Bereavement Coordinator
- ☒ Spiritual Counselor

**Date:** 12/30/2025

**Electronically signed by:** TRACY KING RN

Patient: BARON BROWN  
Medical Record: #107145

# SPIRITUAL ASSESSMENT

## SPIRITUAL ASSESSMENT

Date/Time in: 1/2/2026 9:00:00 AM  
Date/Time out: 1/2/2026 9:20:00 AM  
Location/address: 828 The Heights Dr Apt B Fort Worth, TX 76112-0001  
Name/relationship of person providing information: patient

### Response to Terminal Prognosis

• *Acceptance*

☒ Patient

### Pain Assessment

Pain rating: 0  
Pain scale used: Verbal Scale

### Religious Community

• *Affiliation with a religious community*

☒ Patient

### Indicators of Spiritual Strengths

• *Awareness of/sustained by the transcendent*

☒ Patient

• *Has a sense of meaning, purpose, belonging*

☒ Patient

• *Spiritually at peace*

☒ Patient

• *Trusting, hopeful*

☒ Patient

• *Belief in life after death/eternal life*

☒ Patient

• *Able to receive love/support from others*

☒ Patient

• *Able to love and forgive others*

☒ Patient

• *Practices prayer, worship, and/or meditation*

☒ Patient

• Ability to love and forgive self

☒ Patient

• Finds strength in sacred passages

☒ Patient

**Comments related to spiritual strengths:** Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer.

Indicators of Spiritual Concerns/Spiritual Distress

**Teaching and other interventions provided during visit (include patient/family's response to teaching/interventions):** Educated patient on the role of the chaplain, support and resources available to him/her and the family. Patient affirms understanding verbally and is accepting of future chaplain visits for socialization, scripture and prayer.

Communication and Comments

**Comments related to Plan of Care and overall impressions:** Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer.

**Plan for next visit:** Provide spiritual support through socialization, scripture and prayer

**Date:** 1/2/2026

**Electronically signed by:** BRETT BRATCHER M.DIV

Patient: **BARON BROWN**  
Medical Record: **#107145**

# PSYCHOSOCIAL ASSESSMENT

## PSYCHOSOCIAL ASSESSMENT

Date/Time in: 1/2/2026 12:00:00 PM  
Date/Time out: 1/2/2026 12:59:00 PM  
Others present during the visit caregiver

### Pain

Pain Rating: 1  
Pain scale used: Verbal Scale

### Concrete Needs/Referrals

- ☒ Funeral planning/final arrangements
- ☒ Advance directives

### Communication and Comments

**Comments related to plan of care and overall impressions:** Understanding Goals

Describe the patient/family’s understanding of the patient’s illness, stated concerns, & end-of-life goals:  
Mr. Brown demonstrates an understanding of his cardiac diagnosis and acknowledges that his condition is chronic and progressive. He verbalizes acceptance of hospice services for comfort-focused care. Primary goals include remaining at home, maintaining comfort, and minimizing hospitalizations. No acute concerns expressed at this time. Family appears supportive of comfort-oriented goals.

Hospice Death and Dying

Describe the patient/family’s understanding of hospice, pain and symptom management, and the death and dying process:  
Patient and family verbalize understanding that hospice focuses on comfort, symptom management, and quality of life rather than curative treatment. They understand that hospice will assist with pain and symptom control, nursing oversight, and emotional support. Understanding of the death and dying process is basic but appropriate. Ongoing education to be provided as condition progresses.

Pain

Pain Rating: Denies pain at time of assessment  
Pain scale used: Numeric Rating Scale (0–10)  
Location/Description of pain: No pain reported  
Notified nurse of any pain present: Yes – nurse notified that patient denies pain at this time

Advance Directives

Power of Attorney – Describe: Not identified at this time

Living Will – Describe: Not reported

Do Not Resuscitate – Describe: Patient is currently Full Code (DNR not in place)

Organ/Body Donor – Describe: Not reported

State Guardianship – Describe: None

Surrogate – Describe: Not formally designated

Funeral Plans

Describe patient/family’s funeral plans: Funeral arrangements have not yet been discussed or finalized. Patient and family are aware that hospice social work can assist with planning when ready.  
Funeral home name: Undecided

Address: N/A

#### Social / Cultural

Describe the patient's history and background:

Mr. Brown is a 67-year-old male with a history of employment prior to retirement. No military service reported. Cultural or religious preferences not identified at this time. Patient reports valuing independence and spending time in his home environment. No immigration history or recent significant losses reported.

#### Living Situation / Support System

Describe the patient's current living situation and support system:

Patient resides at home and is receiving Routine Home Care hospice services. Family members are involved and provide emotional and practical support. Current support system appears adequate at this time.

#### Family Dynamics / Impact of Loss

Describe the family dynamics and the impact that the patient's death will have on the family:

Family appears supportive and involved in patient care. Anticipated loss is expected to have a significant emotional impact. Family demonstrates concern and willingness to honor patient's wishes.

#### Psychosocial

Describe the patient/family's mental status and coping ability:

Patient is alert, oriented, and able to engage in conversation. Mood appears appropriate to situation. Coping skills appear adequate at this time. Family demonstrates appropriate coping with signs of anticipatory grief.

#### Financial

Describe the patient/family's financial resources and concerns:

No immediate financial concerns reported. Hospice benefits are active. Family did not express concerns related to cost of care at this time.

#### Concrete Needs / Referrals

Spiritual counselor/caregiver: Not requested at this time

Family counseling: Not requested at this time

Community social services: None identified currently

Meals on Wheels: Not requested

Lifeline: Not requested

Transportation: Not needed at this time

Funeral planning/final arrangements: Education offered; family will notify team when ready

#### Communication and Comments

Communication with other team members:

Hospice RN notified on 01/12/2026 of patient's denial of pain and psychosocial assessment findings.

Comments related to plan of care and overall:

Patient remains appropriate for Routine Home Care hospice services. Focus remains on comfort, symptom monitoring, and ongoing psychosocial support. Social work will continue to monitor coping, advance directive status, and provide education and anticipatory grief support as needed.

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**Date:** 1/2/2026

**Electronically signed by:** ASHLEY DAVIS LMSW

Patient: BARON BROWN  
Medical Record: #107145

Bereavement Risk Assessment

Date/Time In 12/31/2025 3:20:00 PM  
Date/Time Out 12/31/2025 3:40:00 PM  
Risk Assessment Grid

|                                                                        |                                   |
|------------------------------------------------------------------------|-----------------------------------|
| Caregiver                                                              | Khesia Brown (primary - Daughter) |
| 1: Possible alcohol/substance abuse                                    | <input type="checkbox"/>          |
| 1: Neglect of appearance                                               | <input type="checkbox"/>          |
| 1: History of mental or emotional health concerns                      | <input type="checkbox"/>          |
| 1: History of marked dependence on patient                             | <input type="checkbox"/>          |
| 1: History of suicide ideation or intent                               | <input type="checkbox"/>          |
| 1: Age less than 18                                                    | <input type="checkbox"/>          |
| 1: Absence of meaningful spiritual support                             | <input type="checkbox"/>          |
| 1: Death means loss of constant companion or emotional support         | <input type="checkbox"/>          |
| 1: Death will result in loss of financial provisions/financial support | <input type="checkbox"/>          |
| 1: Death means change or loss of role or function                      | <input type="checkbox"/>          |
| 1: Unable to share feelings                                            | <input type="checkbox"/>          |
| 1: Difficulty making decisions                                         | <input type="checkbox"/>          |
| 1: Ambivalent relationship with the patient                            | <input type="checkbox"/>          |
| 1: Death means loss of meaning/purpose in life                         | <input type="checkbox"/>          |
| 1: Communication difficulties                                          | <input type="checkbox"/>          |
| 1: Cultural issues present                                             | <input type="checkbox"/>          |
| 1: Inflexible thinking                                                 | <input type="checkbox"/>          |
| 1: Dependent family members/children living in patient's home          | <input type="checkbox"/>          |
| 1: Unfinished business with patient                                    | <input type="checkbox"/>          |
| 1: Sense of hopelessness                                               | <input type="checkbox"/>          |
| 1: Signs of spiritual distress                                         | <input type="checkbox"/>          |
| 1: Estranged or isolated from support system                           | <input type="checkbox"/>          |
| 1: Inadequate or negative coping skills                                | <input type="checkbox"/>          |
| 1: Concurrent life crisis                                              | <input type="checkbox"/>          |
| 1: Multiple or recent losses                                           | <input type="checkbox"/>          |
| 1: Unprepared for death/strong denial prior to death                   | <input type="checkbox"/>          |
| 1: Difficulty coping with past losses                                  | <input type="checkbox"/>          |

Patient: **BARON BROWN**  
Medical Record: **#107145**

VERBAL PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Verbal Physician Initial Certification of Terminal Illness

|                              |                                                              |
|------------------------------|--------------------------------------------------------------|
| Start of Care Date:          | 12/30/2025                                                   |
| Benefit Period:              | First 90 Days                                                |
| Certification Period - From: | 12/30/2025                                                   |
| Certification Period - To:   | 3/29/2026                                                    |
| Primary Diagnosis:           | Athscl heart disease of native coronary artery w/o ang pctrs |
| ICD-10 Code:                 | I25.10                                                       |

Hospice Physician Initial Certification of Terminal Illness

Verbal Certification:

On 12/30/2025

Dr. DIEGO F RESTREPO MD

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date: 12/30/2025

Physician Signature / Date:

Attending Physician Initial Certification of Terminal Illness

Verbal Certification:

On 12/30/2025

Dr. DIEGO RESTREPO

certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Verbal Certification Received By:

Electronically signed by: TRACY KING RN

Date: 12/30/2025

Physician Signature / Date:



Beneficiary: Brown Baron L  
DOD:

Medicare ID: 9W00K37ER02

Sex: Male

DOB: 10/26/1958

## Beneficiary Information

Beneficiary Last Name:\*

Brown

Beneficiary First Name:\*\*

Baron

Medicare ID: \*

9W00K37ER02

Beneficiary Birth Date:\*\*

10/26/1958

Beneficiary Name Suffix:

Optional Fields for Requesting Historical Data Using Date Range

Date From:

Date To:

## Eligibility

### Part A Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2005

Termination Date:

### Part B Eligibility

Beneficiary insured due to age OASI (Old-Age and Survivors Insurance)

Effective Date:

01/01/2005

Termination Date:

### Part B Immunosuppressive Drug Eligibility

Effective Date:

Termination Date:

### Inactive Periods

Effective Date:

Termination Date:

Reason:

## Beneficiary Address

Address Line 1:

7425 Kell Dr

Address Line 2:

City:

Fort Worth

State:

TX

Zip:

76112-5909

## End Stage Renal Disease (ESRD)

Coverage Period Effective Date:

Coverage Period End Date:

Dialysis Start Date:

Dialysis End Date:

Transplant Effective Date:

## Additional Information

### MBI End Date

End Date:

## Audiology Screening

### HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

|       |            |
|-------|------------|
| 92550 | 07/01/2023 |
| 92552 | 07/01/2023 |
| 92553 | 07/01/2023 |
| 92555 | 07/01/2023 |
| 92556 | 07/01/2023 |
| 92557 | 07/01/2023 |
| 92562 | 07/01/2023 |
| 92563 | 07/01/2023 |
| 92565 | 07/01/2023 |
| 92567 | 07/01/2023 |
| 92568 | 07/01/2023 |
| 92570 | 07/01/2023 |

**HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance**

|       |            |            |
|-------|------------|------------|
| 92571 | 07/01/2023 |            |
| 92572 | 07/01/2023 |            |
| 92575 | 07/01/2023 |            |
| 92576 | 07/01/2023 |            |
| 92577 | 07/01/2023 |            |
| 92579 | 07/01/2023 |            |
| 92582 | 07/01/2023 |            |
| 92583 | 07/01/2023 |            |
| 92584 | 07/01/2023 |            |
| 92587 | 07/01/2023 | 07/01/2023 |
| 92588 | 07/01/2023 | 07/01/2023 |
| 92601 | 07/01/2023 |            |
| 92602 | 07/01/2023 |            |
| 92603 | 07/01/2023 |            |
| 92604 | 07/01/2023 |            |
| 92620 | 07/01/2023 |            |
| 92621 | 07/01/2023 |            |
| 92622 | 07/01/2023 |            |
| 92623 | 07/01/2023 |            |
| 92625 | 07/01/2023 |            |
| 92626 | 07/01/2023 |            |
| 92627 | 07/01/2023 |            |
| 92640 | 07/01/2023 |            |
| 92651 | 07/01/2023 |            |
| 92652 | 07/01/2023 |            |
| 92653 | 07/01/2023 |            |

**MDPP Coverage**

Start Date:

End Date:

**MDPP Inactive Periods**

Start Date:

End Date:

**MDPP Deductible**

Start Date:

End Date:

Deductible Amount:

## MDPP Co-Insurance

Start Date:

End Date:

Co-Insurance Amount:

## MDPP Usage Information

HCPCS Code Billing NPI Date Of Service

---

# Deductibles/Caps

## Acupuncture

Eligibility begins:

01/21/2020

Technical Sessions Remaining

20

Eligibility begins:

01/21/2020

Professional Sessions Remaining

20

## Part B Deductible

Start Date:

01/01/2025

End Date:

10/31/2025

Deductible Amount:

\$257

## Part B Remaining Deductible

Start Date:

01/01/2025

End Date:

10/31/2025

Deductible Amount:

\$0

## Co-insurance Details

Start Date:

01/01/2025

End Date:

10/31/2025

Co-insurance Amount:  
20%

## **Blood Deductible**

Calendar Year:  
2025  
Number Of Units Remaining:  
3

## **Occupational Therapy Cap**

Calendar Year:  
2025  
Amount Used:  
\$0

## **Physical And Speech Therapy Cap**

Calendar Year:  
2025  
Amount Used:  
\$0

## **Pulmonary Rehabilitation Services**

Calendar Year:  
2025  
Professional Sessions Remaining:  
72  
Technical Sessions Remaining:  
72

## **Cardiac Rehabilitation Services**

Calendar Year:  
2025  
Professional Sessions Used:  
0  
Technical Sessions Used:  
0

## **Intensive Cardiac Rehabilitation Services**

Calendar Year:  
2025  
Professional Sessions Used:  
0  
Technical Sessions Used:

0

**Part B Free Services**      [List of STC Codes](#)

STC Codes:

42,5,67,80,AJ,CO

Value:

0%

Start Date:

01/01/2025

End Date:

10/31/2025

**Mental Health Co-insurance (if different from the plan level Co-insurance)**      [List of STC Codes](#)

STC Codes:

Value:

Start Date:

End Date:

# Preventive

## COVID

**COVID-19 Immunization****Immunization Date HCPC Code Rendering Medicare NPI**

06/01/2021      0031A      1487669446

**Pneumococcal Vaccine (PPV)****Pneumococcal Vaccine****Date of Service HCPC Code Rendering Medicare NPI**

No data available

## Flu

**Flu Vaccination****Vaccination Date HCPC Code Rendering Medicare NPI**

No data available

**Cognitive Assessment and Care Plan Services****Cognitive Assessment and Care Plan Services****Date of Service HCPC Code Rendering Medicare NPI**

No data available

## Smoking Cessation

Number of Sessions in Benefit Period:

8

Initial Session Date:

Benefit Period Sessions Remaining:

8

## Preventive

### HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance

|       |            |            |  |
|-------|------------|------------|--|
| 71271 | 01/01/2021 | 01/01/2021 |  |
| 74263 | 01/01/2025 | 01/01/2025 |  |
| 76706 | 07/01/2007 | 07/01/2007 |  |
| 76977 | 01/01/2005 | 01/01/2005 |  |
| 77078 | 01/01/2005 | 01/01/2005 |  |
| 77080 | 01/01/2005 | 01/01/2005 |  |
| 77081 | 01/01/2005 | 01/01/2005 |  |
| 80061 | 01/01/2005 | 01/01/2005 |  |
| 81528 |            | 10/09/2014 |  |
| 82270 | 01/01/2007 | 01/01/2007 |  |
| 82465 | 01/01/2005 | 01/01/2005 |  |
| 82947 | 01/01/2005 | 01/01/2005 |  |
| 82950 | 01/01/2005 | 01/01/2005 |  |
| 82951 | 01/01/2005 | 01/01/2005 |  |
| 83036 | 01/01/2024 | 01/01/2024 |  |
| 83718 | 01/01/2005 | 01/01/2005 |  |
| 84478 | 01/01/2005 | 01/01/2005 |  |
| G0473 | 01/01/2015 | 01/01/2015 |  |
| G0118 | 01/01/2005 | 01/01/2005 |  |
| G0117 | 01/01/2005 | 01/01/2005 |  |
| G0105 | 01/01/2005 | 01/01/2005 |  |
| G0104 | 01/01/2005 | 01/01/2005 |  |
| G0103 | 01/01/2005 | 01/01/2005 |  |
| G0102 | 01/01/2005 | 01/01/2005 |  |
| G0447 | 11/29/2011 | 11/29/2011 |  |
| G0446 | 11/08/2011 | 11/08/2011 |  |
| G0445 | 11/08/2011 | 11/08/2011 |  |
| G0444 | 10/14/2011 | 10/14/2011 |  |
| G0442 | 10/14/2011 |            |  |
| G0439 | 01/01/2011 |            |  |
| G0438 | 01/01/2011 |            |  |
| 0464U |            | 10/09/2014 |  |
| G0130 | 01/01/2005 | 01/01/2005 |  |
| G0328 | 01/01/2005 | 01/01/2005 |  |

**HCPCS Code Next Professional Date Next Technical Date Remaining Deductible Co-insurance**

G0327 07/01/2021

G0121 01/01/2005 01/01/2005

**Hepatitis Screening****Date Of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance****HIV/PrEP - Pre-Exposure Prophylaxis for HIV Prevention****Date of Service HCPCS Code Provider or Rendering NPI Deductible Co-insurance**

No data available

## Plan Coverage

**Medicare Advantage**

Plan Type:

Health Maintenance Organization (HMO) Medicare Risk

Enrollment Date:

07/01/2025

Disenrollment Date:

Contract Number:

H0609

Contract Name:

UNITEDHEALTHCARE BENEFITS OF TEXAS, INC.

Plan Number:

061

Plan Name:

AARP Medicare Advantage Extras from UHC TX-27

Address Line 1:

9800 Health Care Lane MN006-W500

Phone Number:

8006434845

Address Line 2:

City:

Minnetonka

State:

MN

Zip Code:

55343

Website

UHC.com/Medicare

Bill Code :

C



## VBID Model Hospice Benefit Component Links

[Hospice Benefit Component participating plans](#)

[Directions for submitting claims for plans supporting the Hospice Benefit Component of the Model](#)

## Medicare Part D

Enrollment Date:

07/01/2025

Disenrollment Date:

Contract Number:

H0609

Contract Name:

UNITEDHEALTHCARE BENEFITS OF TEXAS, INC.

Plan Number:

061

Plan Name:

AARP Medicare Advantage Extras from UHC TX-27

Address Line 1:

9800 Health Care Lane MN006-W500

Phone Number:

8006434845

Address Line 2:

City:

Minnetonka

State:

MN

Zip Code:

55343

Website

UHC.com/Medicare

Drug Plan :

OT

Enrollment

Y

## Medicare Secondary Payer

### Medicare Secondary Payer

Effective Date:

Termination Date:

Insurer Name:

Policy Number:

Patient Relationship:

Group Number:

Type of Primary Insurance:

Diagnosis Codes:

Address Line 1:

Address Line 2:

City:

State:

Zip Code:

Source Code:

Maintenance Date:

Ongoing Responsibility For Medicals:

# Hospice/HomeHealth

## Home Health Care

Patient Status:

NOA Indicator:

HHEH Start Date:

HHEH End Date:

HHEH DOEBA Date:

HHEH DOLBA Date:

Provider Number:

Provider Number Type:

Contractor Number:

Contractor Name:

HH Certification Start Date(s):

HH Recertification Start Date(s):

## Hospice

Hospice Episodes:

| Effective Date | Termination Date | Start Date (DOEBA) | End Date (DOLBA) | Hospice Days Used | Provider Number | Provider Number Type |
|----------------|------------------|--------------------|------------------|-------------------|-----------------|----------------------|
|----------------|------------------|--------------------|------------------|-------------------|-----------------|----------------------|

## Notices of Election (NOE):

| Date | Election Receipt Date | Provider Number | Provider Number Type | Revocation Code | Election Revocation Date |
|------|-----------------------|-----------------|----------------------|-----------------|--------------------------|
|------|-----------------------|-----------------|----------------------|-----------------|--------------------------|

### Part A Deductible

Start Date:

01/01/2025

End Date:

10/31/2025

Deductible Amount:

\$1676

### Part A Base Deductible Remaining

Start Date:

01/01/2025

End Date:

10/31/2025

Remaining Deductible:

\$1676

### Deductible Remaining By Spell

DOEBA Date DOLBA Date Deductible Amt

No data to display

### Inpatient Spell Dates

Includes Hospital and Skilled Nursing Facility (SNF) Dates

No data to display

## Inpatient Base Summary

| Start Date | End Date   | Full Days Allowed | Full Copayment Days Allowed | Copay Amount |
|------------|------------|-------------------|-----------------------------|--------------|
| 01/01/2025 | 10/31/2025 | 60                | 30                          | \$419        |

## Inpatient Days Remaining (May Also Include Spells)

| DOEBA Date | DOLBA Date | Full Inpatient Days | Full Inpatient Copay Days | Copay Amount |
|------------|------------|---------------------|---------------------------|--------------|
| 01/01/2025 | 10/31/2025 | 60                  | 30                        | \$419        |

## SNF Base Summary

| Start Date | End Date   | SNF Full Days Allowed | SNF Full Copayment Days Allowed | SNF Copay Amount |
|------------|------------|-----------------------|---------------------------------|------------------|
| 01/01/2025 | 10/31/2025 | 20                    | 80                              | \$209.5          |

## SNF Days Remaining (May Also Include Spells)

| DOEBA Date | DOLBA Date | Full SNF Days | Full SNF Copay Days | SNF Copay Amount |
|------------|------------|---------------|---------------------|------------------|
| 01/01/2025 | 10/31/2025 | 20            | 80                  | 209.5            |

## Lifetime Reserve Days

Lifetime Days Allowed:

60

Lifetime Days Remaining :

60

Lifetime Days Allowed:

Lifetime Days Remaining :

Calendar Year:

Copayment Amount Per Day:

## Lifetime Psychiatric Benefit Data

Lifetime Psychiatric Base Days:

190

Lifetime Psychiatric Remaining:

190

## Part A Free Services      List of STC Codes

STC Codes:

42,45

Value:

0%

Start Date:

01/01/2025

End Date:

10/31/2025

# Qualified Medicare Beneficiary (QMB) Program Eligibility

Beneficiaries who are enrolled in the Qualified Medicare Beneficiary (QMB) program are dually eligible for both Medicare and Medicaid. Beneficiaries who are enrolled in the QMB program are not liable for Medicare co-insurance or deductible payments. Note that QMB status may fluctuate for a minority of Beneficiaries. If eligibility results indicate the Beneficiary QMB enrollment has terminated, please verify the patient's QMB status through State online Medicaid eligibility systems or other documentation, including Medicaid Identification cards and documents issued by the State proving the patient qualifies for the QMB program.

## Medicaid Enrollment

State:

TX QMB Plan

Enrollment Date:

11/01/2025

Termination Date:

## Part A Deductible

Start Date:

11/01/2025

End Date:

12/31/2025

Deductible Amount:

\$0

## Inpatient Base Summary

Start Date:

11/01/2025

End Date:

12/31/2025

Full Days Allowed:

60

Full Copay Days Allowed:

30

Copay Amount:

\$0

## Inpatient Days Remaining (May Also Include Spells)

DOEBA Date:

11/01/2025

DOLBA Date:

12/31/2025

Full Inpatient Days:

60

Full Inpatient Copay Days:

30

Copay Amount:

\$0

## **SNF Base Summary**

Start Date:

11/01/2025

End Date:

12/31/2025

SNF Full Days Allowed:

20

SNF Full Copay Days:

80

Copay Amount:

\$0

## **SNF Days Remaining (May Also Include Spells)**

DOEBA Date:

11/01/2025

DOLBA Date:

12/31/2025

Full SNF Days:

20

Full SNF Copay Days:

80

Copay Amount:

\$0

## **Lifetime Reserve**

Lifetime Days Allowed:

60

Lifetime Days Remaining:

60

Start Date:

11/01/2025

End Date:

12/31/2025

Copay Amount:

\$0

## **Part B Deductible**

Start Date:

11/01/2025

End Date:

12/31/2025

Deductible Amount:  
\$0

## **Part B Co-Insurance**

Start Date:  
11/01/2025

End Date:  
12/31/2025

Co-Insurance Amount:  
\$0

## Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

### Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Baron Brown

, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

**Admission Information:** The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

**After Hours Access/Inpatient Care:** A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

**Clinical Records:** It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

### Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 9W00-K37-FR02 Effective Date: 01-01-2005

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: \_\_\_\_\_ Effective Date: \_\_\_\_\_

☐ Bill Primary Insurance: \_\_\_\_\_ % Insurance Co: \_\_\_\_\_

Beneficiary/Pre-Auth Numbers: \_\_\_\_\_ Effective Date: \_\_\_\_\_

☐ Bill Secondary Insurance: \_\_\_\_\_ % Insurance Co: \_\_\_\_\_

Beneficiary/Pre-Auth Numbers: \_\_\_\_\_ Effective Date: \_\_\_\_\_

Benefits are: ☐ Self-pay ☐ Charity or ☐ Other \_\_\_\_\_

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.



## Hospice Informed Consent/Election of Hospice Benefit

### Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST  
☐ Other: \_\_\_\_\_

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to [ConsumerProtection@tdi.texas.gov](mailto:ConsumerProtection@tdi.texas.gov).

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

## Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: \_\_\_\_\_ Phone: \_\_\_\_\_

### Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

## Hospice Informed Consent/Election of Hospice Benefit

### Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

x Baron Brown  
Beneficiary/Patient Signature

12-30-25  
Date

\_\_\_\_\_  
Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship to Patient

\_\_\_\_\_  
Reason patient is unable to sign

[Signature]  
Hospice Agency Representative Signature

12-30-25  
Date

### Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Baron Brown, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 12-30-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

### Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

## Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

### Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

### Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: [www.acentraqio.com](http://www.acentraqio.com), phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

### Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
  - Provided by the designated Hospice.
  - Provided by another hospice under arrangements made by the designated Hospice.
  - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

### Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☐ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) \_\_\_\_\_

Office Address: \_\_\_\_\_

## Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

### Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ N/A Patient is non-Medicare

☐ N/A I was transferred from \_\_\_\_\_ on \_\_\_\_\_

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice with the effective date of election to begin on 12-30-25

I am electing the (1<sup>st</sup>, 2<sup>nd</sup>, 3<sup>rd</sup>, 4<sup>th</sup>, etc.) 1<sup>st</sup> benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

x Brian H. Brown  
Beneficiary/Patient Signature

12-30-25  
Date

\_\_\_\_\_  
Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship to Patient

\_\_\_\_\_  
Reason patient is unable to sign

[Signature]  
Hospice Agency Representative Signature

12-30-25  
Date

# Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: \_\_\_\_\_. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.



Signature of Beneficiary/Patient's Authorized Representative

12-30-25

Date

Representative's Relationship/Reason Patient Unable to Sign

Date



Signature of Witness/Hospice Representative

12-30-25

Date

The Hospice furnished the addendum on:

Date

## No Primary Caregiver Agreement

I, Baron Brown, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_
4. \_\_\_\_\_

Baron A. Brown

Patient Signature

12-30-25

Date

\_\_\_\_\_  
Witness

\_\_\_\_\_  
Date

## Patient Choice Statement

(To be completed for all new patients/guardians)

I, Baron Brown (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

x Baron H. Brown

12-30-25

Signature of Patient/Guardian

Date

[Signature]

12-30-25

Signature of Hospice Representative

Date



## Hospice Wound Photography Consent

I, Baron Brown hereby consent and so authorize the taking  
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 12-30-25

Baron K. Brown  
Patient Signature (or other authorized signature)

12-30-25  
Date

\_\_\_\_\_  
Witness Signature

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship, if other than patient

## Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

### Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

### Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

x Dawn L. Brown

Beneficiary/Patient/Authorized Representative Signature

12-30-25

Date

[Signature]

Hospice Agency Representative Signature

12-30-25

Date



HOMAGE  
—HOSPICE—

Homage Hospice  
8204 Elmbrook Drive, Suite 276  
Dallas, Texas 75247

Office Hours:  
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

## Medical Records Release Form

Patient Name Baron Brown Birth Date 10-26-58 SS# 453-21-8528  
Address 828 The Heights Dr. Apt 13  
City Fort Worth State TX Zip 76112  
Home Phone \_\_\_\_\_ Business Ph \_\_\_\_\_ Cell 817-507-8133

### Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

### Medical Records Requested

For the past 5 year:

\*\*Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.\*\*

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:

Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)  
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)  
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Homage Hospice in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

x Baron L. Brown

Patient/Authorized Signature

12-30-25

Date

## Medicare Secondary Payer Information and Screening

Medicare Secondary Payer (MSP) is a term used when Medicare is not responsible for paying first on a healthcare claim. MSP provisions apply to two broad categories of insurance: **Group Health Plan (GHP)** and **Non-Group Health Plan (NGHP)**. When certain conditions are met, Medicare is the secondary payer to:

- **GHP** for services provided to the following groups of Medicare beneficiaries: the Working Aged, Disabled individuals, and individuals with End-Stage Renal Disease (ESRD).
- **NGHP** encompasses three different types of insurance: liability, no-fault, and workers' compensation.

Medicare must be made aware of other primary insurance to ensure Medicare does not mistakenly pay as primary. This screening questionnaire is used to assist in the determination of a primary payer source other than Medicare. If this determination is made, then the hospice will bill the primary payer source initially, and Medicare is later billed as a secondary payer in consideration of any remaining balance.

The admitting clinician should complete the Medicare Secondary Payer Screening Questionnaire on the following page based on the patient responses and return to the office for further processing.

### Resources

<https://www.cms.gov/medicare/coordination-benefits-recovery/overview/secondary-payer>

<https://dominoapps.palmettogba.com/palmetto/webTool.nsf/vTool/msp>

[https://www.cgsmedicare.com/jc/claims/msp\\_tool.html#tool](https://www.cgsmedicare.com/jc/claims/msp_tool.html#tool)

## Medicare Secondary Payer

## Screening Questionnaire

Patient Name: Baron Brown

The questions below are to assist in determining if Medicare is the primary or secondary payer source for the patient. If the answers to any of the questions below are **Yes**, then that **payer source** may be the **primary payer** and **Medicare the secondary payer**.

Is the patient receiving benefits from any of the following programs?

Black Lung Benefits (BL)

☐ Yes ☒ No

Department of Veteran Affairs (DVA)

☐ Yes ☒ No

Government Research Grant

☐ Yes ☒ No

Workers' Compensation

☐ Yes ☒ No

Was the illness/injury due to non-work-related accident? ☐ Yes ☒ No

What type of accident caused the illness/injury? ☐ Motor Vehicle ☐ Non-Motor Vehicle

Is the patient receiving treatment for an injury or illness which another party may be liable? ☐ Yes ☒ No

Is the patient entitled to Medicare based on age? ☒ Yes ☐ No

Is the patient entitled to Medicare based on disability? ☐ Yes ☒ No

Is the patient entitled to Medicare based on ESRD? ☐ Yes ☒ No

Does the patient have a group health insurance plan through an employer, through another family member, or through a Consolidated Omnibus Budget Reconciliation Act of 1985 (COBRA) coverage plan?

☐ Yes ☒ No

☒ Form was returned to the office for review.

Signature of Hospice Staff Completing the Form: [Signature]

Date: 12-30-25

Signature of Hospice Staff Completing the Review/Determination: \_\_\_\_\_

Date: \_\_\_\_\_

Determination - Medicare is: ☒ Primary ☐ Secondary ☐ N/A \_\_\_\_\_

## Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

### Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Baron Brown

, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

**Admission Information:** The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

**After Hours Access/Inpatient Care:** A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

**Clinical Records:** It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

### Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 9W00-K37-FR02 Effective Date: 01-01-2005

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: \_\_\_\_\_ Effective Date: \_\_\_\_\_

☐ Bill Primary Insurance: \_\_\_\_\_ % Insurance Co: \_\_\_\_\_

Beneficiary/Pre-Auth Numbers: \_\_\_\_\_ Effective Date: \_\_\_\_\_

☐ Bill Secondary Insurance: \_\_\_\_\_ % Insurance Co: \_\_\_\_\_

Beneficiary/Pre-Auth Numbers: \_\_\_\_\_ Effective Date: \_\_\_\_\_

Benefits are: ☐ Self-pay ☐ Charity or ☐ Other \_\_\_\_\_

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

## Hospice Informed Consent/Election of Hospice Benefit

### Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST  
☐ Other: \_\_\_\_\_

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to [ConsumerProtection@tdi.texas.gov](mailto:ConsumerProtection@tdi.texas.gov).

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

## Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: \_\_\_\_\_ Phone: \_\_\_\_\_

### Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.



## Hospice Informed Consent/Election of Hospice Benefit

### Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

x Baron Brown  
Beneficiary/Patient Signature

12-30-25  
Date

\_\_\_\_\_  
Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship to Patient

\_\_\_\_\_  
Reason patient is unable to sign

MJ  
Hospice Agency Representative Signature

12-30-25  
Date

### Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Baron Brown, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 12-30-25 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

### Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

## Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

### Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

### Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: [www.acentraqio.com](http://www.acentraqio.com), phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

### Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
  - Provided by the designated Hospice.
  - Provided by another hospice under arrangements made by the designated Hospice.
  - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

### Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☐ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) \_\_\_\_\_

Office Address: \_\_\_\_\_

## Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

### Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☐ N/A Patient is non-Medicare

☐ N/A I was transferred from \_\_\_\_\_ on \_\_\_\_\_

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice with the effective date of election to begin on 12-30-25

I am electing the (1<sup>st</sup>, 2<sup>nd</sup>, 3<sup>rd</sup>, 4<sup>th</sup>, etc.) 1<sup>st</sup> benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

x Brian H. Brown  
Beneficiary/Patient Signature

12-30-25  
Date

\_\_\_\_\_  
Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship to Patient

\_\_\_\_\_  
Reason patient is unable to sign

[Signature]  
Hospice Agency Representative Signature

12-30-25  
Date

# Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: \_\_\_\_\_. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.



Signature of Beneficiary/Patient's Authorized Representative

12-30-25

Date

Representative's Relationship/Reason Patient Unable to Sign

Date



Signature of Witness/Hospice Representative

12-30-25

Date

The Hospice furnished the addendum on:

Date

## No Primary Caregiver Agreement

I, Baron Brown, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. \_\_\_\_\_
2. \_\_\_\_\_
3. \_\_\_\_\_
4. \_\_\_\_\_

Baron A. Brown

Patient Signature

12-30-25

Date

\_\_\_\_\_  
Witness

\_\_\_\_\_  
Date

## Patient Choice Statement

(To be completed for all new patients/guardians)

I, Baron Brown (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

x Baron H. Brown

12-30-25

Signature of Patient/Guardian

Date

[Signature]

12-30-25

Signature of Hospice Representative

Date

## Hospice Wound Photography Consent

I, Baron Brown hereby consent and so authorize the taking  
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 12-30-25

Baron K. Brown  
Patient Signature (or other authorized signature)

12-30-25  
Date

\_\_\_\_\_  
Witness Signature

\_\_\_\_\_  
Date

\_\_\_\_\_  
Relationship, if other than patient

## Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

### Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

### Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

x Dawn L. Brown

Beneficiary/Patient/Authorized Representative Signature

12-30-25

Date

[Signature]

Hospice Agency Representative Signature

12-30-25

Date





HOMAGE  
—HOSPICE—

Homage Hospice  
8204 Elmbrook Drive, Suite 276  
Dallas, Texas 75247

Office Hours:  
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

## Medical Records Release Form

Patient Name Baron Brown Birth Date 10-26-58 SS# 453-21-8528  
Address 828 The Heights Dr. Apt 13  
City Fort Worth State TX Zip 76112  
Home Phone \_\_\_\_\_ Business Ph \_\_\_\_\_ Cell 817-507-8133

### Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

### Medical Records Requested

For the past 5 year:

\*\*Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.\*\*

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:

Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)  
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)  
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Homage Hospice in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

x Baron L. Brown

Patient/Authorized Signature

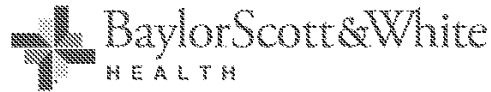
12-30-25

Date

Fax From Bsw Healthcare

Dec 15th, 2025 3:05pm CST

|                   |                        |
|-------------------|------------------------|
| From              | (214) 820 - 0111       |
| To                | (469) 617 - 6998       |
| Result            | Fax Receive Successful |
| Sender Caller ID  | BSW Healthcare         |
| Number of Pages   | 19                     |
| Transmission Time | 9 minutes, 39 seconds  |



## *Confidential Fax Transmission*

---

Baylor University Medical Center  
Health Information Management  
3500 Gaston Avenue  
Dallas Texas [44] 75246  
Phone: 214-820-6037  
Fax: 214-818-6782

**Date:** 12/15/25

**To:** NEW REQUESTER

**Phone Number:**

**Fax Number:**

**Release ID:** 971947798

**Subject:** Request for Medical Records

**Comments:** Copies of the records you requested are attached.

---

**STATEMENT OF CONFIDENTIALITY:** The information contained in this facsimile message may be confidential and/or privileged. This information is intended to be reviewed initially only by the individual named above. If the reader of this transmittal page is not the intended recipient or a representative of the intended recipient, you are hereby notified that any review, dissemination, or copying of this fax or the information contained herein is prohibited. If you have received this fax in error, please immediately notify Scott & White at 254.724.7600 or 866.218.6920. Thank you.

| Baylor University Medical Center |                            | FACESHEET<br>ADMISSION/REGISTRATION RECORD |                                        |
|----------------------------------|----------------------------|--------------------------------------------|----------------------------------------|
| <b>PATIENT INFORMATION</b>       |                            | <b>MEDICAL RECORD #:</b> 10479960          |                                        |
| NAME:                            | BROWN, BARON               | DOB: 10/26/1958                            | AGE: 61                                |
| ADDRESS 1:                       | 4401 DISRAELI DR           | GENDER: MALE                               | SOC. SECURITY #: 453-21-8528           |
| ADDRESS 2:                       | APT 5303                   | RACE: BLACK                                | MARITAL STATUS: SINGLE                 |
| CITY, STATE ZIP:                 | FORT WORTH, TX 76119       | ETHNICITY: NOT OF HISPANIC ORIGIN          |                                        |
| PHONE:                           | (817) 507-8133             | LANGUAGE: ENGLISH                          |                                        |
| ALIAS/MAIDEN NAME:               |                            | RELIGION: CHRISTIAN                        |                                        |
| EMAIL:                           | BARON833@GMAIL.COM         | EMERGENCY CONTACT: KEYS, KATHERINE         |                                        |
| EMPLOYER:                        | SELF-EMPLOYED              | RELATIONSHIP TO PATIENT: SIGNIFICANT OTHER |                                        |
| ADDRESS 1:                       |                            | PHONE: (214) 499-5705                      |                                        |
| ADDRESS 2:                       |                            |                                            |                                        |
| CITY, STATE ZIP:                 |                            |                                            |                                        |
| PHONE:                           |                            |                                            |                                        |
| <b>VISIT INFORMATION</b>         |                            | <b>ACCOUNT/VISIT#:</b> 62744265            |                                        |
| ADMIT DATE/TIME:                 | 01/31/2019 09:01 AM        | VISIT TYPE: EMERGENCY                      |                                        |
| LOCATION:                        | BUMC-EDL-                  | ADMIT TYPE: EMERGENCY                      |                                        |
| ACCOMMODATION:                   |                            | PRIVACY STATUS: ROUTINE/STANDARD           |                                        |
| SERVICE CODE:                    | EMERGENCY MEDICINE         |                                            |                                        |
| VISIT REASON:                    | DEPRESSION                 |                                            |                                        |
| ACCIDENT TYPE:                   |                            | ACCIDENT DATE/TIME:                        |                                        |
| DISCHARGE DATE/TIME:             | 01/31/2019 02:00 PM        | OCCURRENCE DATE: 01/31/2019                | OCCURRENCE CODE: 11                    |
|                                  |                            | 10/26/1958                                 | A1                                     |
| ADMITTING PHYSICIAN:             |                            | ATTENDING PHYSICIAN:                       | CONSULTING PHYSICIAN:                  |
|                                  |                            | SEUNG DAMRONG, JASON                       |                                        |
| PRIMARY CARE PHYSICIAN:          |                            | REFERRING PHYSICIAN:                       |                                        |
| SCHIRANZ, DAMON ASHLEY           |                            |                                            |                                        |
| <b>GUARANTOR INFORMATION</b>     |                            |                                            |                                        |
| NAME:                            | BROWN, BARON               | DOB: 10/26/1958                            | RELATIONSHIP TO PATIENT: SELF/PATIENT  |
| HOME PHONE:                      | (817) 507-8133             | SOC. SECURITY #: 453-21-8528               |                                        |
| BUSINESS PHONE:                  |                            |                                            |                                        |
| EMPLOYER:                        | SELF-EMPLOYED              |                                            |                                        |
| EMPLOYER PHONE:                  |                            |                                            |                                        |
| <b>INSURANCE INFORMATION</b>     |                            |                                            |                                        |
| CODE:                            | PRIMARY                    | SECONDARY                                  | TERTIARY                               |
| PLAN:                            | MCWL                       |                                            |                                        |
| GROUP:                           | MEDICARE WELL CARE         |                                            |                                        |
| GROUP #:                         | WELL CARE ACCESS           |                                            |                                        |
| POLICY #:                        | 80840                      |                                            |                                        |
| ADDRESS 1:                       | 9899560                    |                                            |                                        |
| ADDRESS 2:                       | PO BOX 31372               |                                            |                                        |
| CITY, STATE ZIP:                 | TAMPA, FL 33631            |                                            |                                        |
| INSURED'S NAME:                  | BROWN, BARON               |                                            |                                        |
| INSURED'S DOB:                   | 10/26/1958                 |                                            |                                        |
| INSURED'S SOC. SECURITY #:       | 453-21-8528                |                                            |                                        |
| RELATIONSHIP TO INSURED:         | SELF/PATIENT               |                                            |                                        |
| AUTHORIZATION #:                 |                            |                                            |                                        |
| PRECERTIFICATION PHONE:          |                            |                                            |                                        |
| PRECERTIFICATION DAYS:           |                            |                                            |                                        |
| INS. VERIFICATION PHONE:         | (866) 530-9495             |                                            |                                        |
| <b>BROWN, BARON</b>              |                            |                                            |                                        |
| PRINTED: 06/22/2020              | MEDICAL RECORD #: 10479960 | ACCOUNT/VISIT #: 62744265                  | ADMIT DATE/TIME: 01/31/2019 / 09:01 AM |

**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LOC: BEDL

BILL#: 62744265

DOB: 10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER: 5131007483

Acetaminophen

COLLECTED: 01/31/2019 10:00

| TEST NAME     | RESULT | ABN FL | RANGES      | UNITS | S |
|---------------|--------|--------|-------------|-------|---|
| Acetaminophen | <2.0   | L      | 10.0 - 20.0 | ug/mL | F |

A = Abnormal

L = Low

H = High

AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT.PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BEDL--

ADM.DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:31 PM

Page: 1/-

**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LOC: BED1

BILL#: 62744265

DOB: 10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER : 6131007483

Alcohol

COLLECTED: 01/31/2019 10:00

| TEST NAME | RESULT   | ABN FL | RANGES   | UNITS | S |
|-----------|----------|--------|----------|-------|---|
| Alcohol   | None Det |        | None Det | mg/dL | F |

A = Abnormal

L = Low

H = High

AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT. PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BED1--

ADM. DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:31 PM

Page: 1/-

**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LOC: BED1

BILL#: 62744265

DOB: 10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER: 5131016460

CBC With Differential

COLLECTED: 01/31/2019 10:00

| TEST NAME                                                        | RESULT    | ABN FL | RANGES      | UNITS | S |
|------------------------------------------------------------------|-----------|--------|-------------|-------|---|
| WBC                                                              | 8.1       |        | 4.5 - 11.0  | K/uL  | F |
| RBC                                                              | 5.53      |        | 4.50 - 6.00 | M/uL  | F |
| Hemoglobin                                                       | 15.6      |        | 13.5 - 18.0 | g/dL  | F |
| Hematocrit                                                       | 46.0      |        | 40.0 - 52.0 | %     | F |
| MCV                                                              | 83.2      |        | 80.0 - 99.0 | fL    | F |
| MCH                                                              | 28.2      |        | 27.0 - 33.0 | pg    | F |
| MCHC                                                             | 33.9      |        | 32.0 - 36.5 | %     | F |
| Platelet Count                                                   | 199       |        | 140 - 440   | K/uL  | F |
| RDW-SD                                                           | 40.3      |        | 37.0 - 51.0 | fL    | F |
| RDW-CV                                                           | 13.5      |        | 10.0 - 14.5 | %     | F |
| MPV                                                              | 11.3      |        | 8.5 - 12.0  | fL    | F |
| Seg                                                              | 80        |        |             | %     | F |
| Lymphocytes                                                      | 15        |        |             | %     | F |
| Absolute Seg                                                     | 6.48      |        | 2.03 - 8.25 | K/uL  | F |
| Absolute Lymphocytes                                             | 1.22      |        | 0.90 - 4.95 | K/uL  | F |
| WBC Mixed Cells                                                  | 5         |        |             | %     | F |
| This component consists of Monocytes, Eosinophils, and Basophils |           |        |             |       |   |
| Absolute WBC Mixed Cells                                         | 0.40      |        | 0.20 - 1.10 | K/uL  | F |
| This component consists of Monocytes, Eosinophils, and Basophils |           |        |             |       |   |
| ANC                                                              | 6.48      |        | 2.07 - 8.80 | K/uL  | F |
| Manual Differential                                              | AUTO DIFF |        |             |       | F |

A = Abnormal

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AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT.PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BED1--

ADM.DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:21 PM

Page: 1/-



Baylor University Medical Center at Dallas

**NAME:** BROWN, BARON  
**DOB:** 10/26/1958  
**VISIT#:** 62744265  
**MRN#:** 10479960

**ADM:** 01/31/2019  
**AGE:** 60y  
**SEX:** Male

This is your new discharge medication list.

1. We made this list from the medicine you told us you take at home and any new medicine your doctor added or changed during your stay.
2. Please tell your primary doctor you were in the hospital and:
  - a. Give him/her a copy of this list
  - b. Throw away any other medicine lists you have, DO NOT throw away the actual medicine
  - c. DO NOT take any other medicine if it is not on this list until you talk to your doctor and he/she says you may take it
3. If you have any questions, please contact your primary doctor right away.

**NO ACTIVE MEDICATIONS**

Created: 1/31/2019 3:06:35 PM

Page: 1 of 1

|                                                                                                                                                                  |                                                                                                                         |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------|
| <br>Visit 62744265 31-Jan-2019<br>BROWN, BARON<br>MRN 10479960 10/26/1958 60y | <b>Baylor Health Care System</b><br><br>SC60347 (Rev05/14 ver 1.0)<br><b>DISCHARGE (HOME) MEDICATION RECONCILIATION</b> |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------|



**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LOC: BEDL

BILL#: 62744265

DOB: 10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER : 5131007484

Drug Abuse Screen Urine

COLLECTED: 01/31/2019 10:00

| TEST NAME                                                                                                                                                                                                                                                                                                                                                                                                        | RESULT                | ABN FL | RANGES   | UNITS | S |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|--------|----------|-------|---|
| UR Amphetamines Screen                                                                                                                                                                                                                                                                                                                                                                                           | NEG                   |        | Negative |       | F |
| Urine Amphetamines Cutoff 1000 ng/mL                                                                                                                                                                                                                                                                                                                                                                             |                       |        |          |       |   |
| UR Barbiturate Screen                                                                                                                                                                                                                                                                                                                                                                                            | NEG                   |        | Negative |       | F |
| Urine Barbiturate Cutoff 200 ng/mL                                                                                                                                                                                                                                                                                                                                                                               |                       |        |          |       |   |
| UR Benzodiazepine Screen                                                                                                                                                                                                                                                                                                                                                                                         | NEG                   |        | Negative |       | F |
| Urine Benzodiazepines Cutoff 200 ng/mL                                                                                                                                                                                                                                                                                                                                                                           |                       |        |          |       |   |
| UR Cannabinoid Screen                                                                                                                                                                                                                                                                                                                                                                                            | NEG                   |        | Negative |       | F |
| Urine Cannabinoids Cutoff 50 ng/mL                                                                                                                                                                                                                                                                                                                                                                               |                       |        |          |       |   |
| UR Cocaine Screen                                                                                                                                                                                                                                                                                                                                                                                                | NEG                   |        | Negative |       | F |
| Urine Cocaine Cutoff 300 ng/mL                                                                                                                                                                                                                                                                                                                                                                                   |                       |        |          |       |   |
| UR Methadone Screen                                                                                                                                                                                                                                                                                                                                                                                              | NEG                   |        | Negative |       | F |
| Urine Methadone Cutoff 300 ng/mL                                                                                                                                                                                                                                                                                                                                                                                 |                       |        |          |       |   |
| UR Opiate Screen                                                                                                                                                                                                                                                                                                                                                                                                 | NEG                   |        | Negative |       | F |
| Urine Opiates Cutoff 300 ng/mL                                                                                                                                                                                                                                                                                                                                                                                   |                       |        |          |       |   |
| UR PCP Screen                                                                                                                                                                                                                                                                                                                                                                                                    | NEG                   |        | Negative |       | F |
| Urine Phencyclidine Cutoff 25 ng/mL                                                                                                                                                                                                                                                                                                                                                                              |                       |        |          |       |   |
| UR Creatinine Random                                                                                                                                                                                                                                                                                                                                                                                             | 495.00                |        |          | mg/dL | F |
| Creatinine Concentration:                                                                                                                                                                                                                                                                                                                                                                                        | Interpretation:       |        |          |       |   |
| < 20 mg/dL                                                                                                                                                                                                                                                                                                                                                                                                       | Dilute urine specimen |        |          |       |   |
| Comment:                                                                                                                                                                                                                                                                                                                                                                                                         | See Below             |        |          |       | F |
| The drugs of abuse for urine is intended for medical use in the clinical management of patients. Positive results have not been subjected to confirmation unless specifically noted. The urine drug concentration must exceed the SAMHSA guidelines to be considered positive. Screening positive results will be confirmed by an alternate method only upon request within 48 hours of the original collection. |                       |        |          |       |   |

A = Abnormal

L = Low

H = High

AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT.PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BED---

ADM.DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:33 PM

Page: 1/-

**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LOC: BEDL

BILL#: 62744265

DOB: 10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER: 6131008213

POC Comp Panel

COLLECTED: 01/31/2019 10:09

| TEST NAME                                                                                                                                                           | RESULT | ABN FL | RANGES     | UNITS  | S |
|---------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------|--------|------------|--------|---|
| POC Sodium                                                                                                                                                          | 141    |        | 128 - 145  | mmol/L | F |
| POC Potassium                                                                                                                                                       | 3.5    | L      | 3.6 - 5.1  | mmol/L | F |
| Hemolysis cannot be detected on whole blood testing.<br>Potassium results are falsely elevated when hemolysis is present. Interpret potassium results with caution. |        |        |            |        |   |
| POC Chloride                                                                                                                                                        | 104    |        | 98 - 108   | mmol/L | F |
| POC Blood Urea Nitrogen                                                                                                                                             | 14     |        | 7 - 22     | mg/dL  | F |
| POC Glucose                                                                                                                                                         | 129    | F      | 73 - 118   | mg/dL  | F |
| POC TCO2                                                                                                                                                            | 26     |        | 18 - 33    | mmol/L | F |
| POC Creatinine                                                                                                                                                      | 1.2    |        | 0.6 - 1.2  | mg/dL  | F |
| POC ALT                                                                                                                                                             | 29     |        | 10 - 47    | U/L    | F |
| POC Alkaline Phosphatase                                                                                                                                            | 60     |        | 53 - 128   | U/L    | F |
| POC AST                                                                                                                                                             | 23     |        | 11 - 38    | U/L    | F |
| POC Calcium                                                                                                                                                         | 9.3    |        | 8.0 - 10.3 | mg/dL  | F |
| POC Total Bilirubin                                                                                                                                                 | 0.9    |        | 0.2 - 1.6  | mg/dL  | F |
| POC Total Protein                                                                                                                                                   | 7.8    |        | 6.4 - 8.1  | g/dL   | F |
| Performed and reviewed by point of care operator.                                                                                                                   |        |        |            |        |   |
| POC Albumin                                                                                                                                                         | 4.4    |        | 3.3 - 5.5  | g/dL   | F |

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AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT.PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BEDL--

ADM.DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:35 PM

Page: 1/-

**BAYLOR UNIVERSITY MEDICAL CENTER**

3500 GASTON AVENUE, DALLAS, TX, 75246 214-820-9999

PATIENT: BROWN, BARON

MRN: 10479960 LCC :BEDL

BILL#: 62744265

DOB :10/26/1958 SEX: M

ORDERED BY: SEUNG DAMRONG, JASON

ORDER : 5131007483

Salicylate

COLLECTED: 01/31/2019 10:00

| TEST NAME  | RESULT | ABN FL | RANGES     | UNITS | S |
|------------|--------|--------|------------|-------|---|
| Salicylate | <5.0   |        | 5.0 - 15.0 | mg/dL | F |

A = Abnormal

L = Low

H = High

AA = Critical

LL = Critical Low

HH = Critical High

KEY FOR RESULTS: R - NEW RESULT \*\* - RESULT WAS MODIFIED AFTER FINAL STATUS SET

ATT.PHYS.: SEUNG DAMRONG, JASON

MRN: 10479960

LOCATION: BEDL--

ADM.DATE: 01/31/2019

PATIENT: BROWN, BARON

PRINTED: 06/22/2020 07:31 PM

Page: 1/-

**BAYLOR UNIVERSITY MEDICAL CENTER**

PATIENT NAME: Brown, Baron Lloyd  
MRN / ACCOUNT #: 10479960 / 62744265  
DOB / AGE / SEX: 10/26/1958 / 61Y / M

ADMIT DATE: 01/31/2019 09:01 AM  
DICTATION DATE/TIME:  
PROVIDER: Seungdamrong, Jason

**ED DISPOSITION SUMMARY**

Discharge Summary  
Baylor University Medical Center  
Name: Baron Brown  
Emergency Department  
Age: 60 yrs  
Sex: Male  
DOB: 10/26/1958  
MRN: 10479960  
Arrival: 01/31/2019  
09:01  
Account #: 62744265  
Departure Date: 01/31/2019  
Departure Time: 13:58  
Private MD:  
Outcome: Discharge  
Location: Home  
Condition: Stable  
Chief Complaint: Depression  
Diagnosis: - Depression, Adjustment disorder, unspecified  
Prescriptions:  
Custom Notes:  
Attending Physician: Seungdamrong, Jason, MD  
Private MD:  
Mid Level Provider:  
Orders: Level - Salicylate (Aspirin), Level - Acetaminophen  
(Tylenol), CBC (Complete Blood Count), CMP (Comprehensive Metabolic  
Panel), Level - ETOH (Alcohol) Serum, UDS (Urine Drug Screen), Low  
Risk, Psych Consult - Depression, BP Recheck, LL POC CBC, LL POC  
Comp Panel, GFR AA and Non AA, POC Comp Panel  
Discharge Instruction: Discharge Summary Sheet, Adjustment Disorder,  
Adult, How to Take Your Blood Pressure, Easy-to-Read, Medication  
Reconciliation  
\*\*\*\*\*

**Brown, Baron Lloyd**

Enterprise Patient ID: 1010937371

Medical Record #: 10479960 / Account #: 62744265

ED Disposition Summary

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 1 of 1

**BAYLOR UNIVERSITY MEDICAL CENTER**

PATIENT NAME: Brown, Baron Lloyd  
 MRN / ACCOUNT #: 10479960 / 62744265  
 DOB / AGE / SEX: 10/26/1958 / 61Y / M

ADMIT DATE: 01/31/2019 09:01 AM  
 DICTATION DATE/TIME:  
 PROVIDER: Seungdamrong, Jason

**ED NURSE NOTES**

## Nurse's Notes

Baylor University Medical Center

Name: Baron Brown

Age: 60 yrs

Sex: Male

DOB: 10/26/1958

MRN: 10479960

Arrival Date: 01/31/2019

Time: 09:01

Account#: 62744265

Bed L2

Private MD:

Diagnosis: Depression; Adjustment disorder, unspecified

## Presentation:

01/31

09:05 Transition of care: patient was not received from another setting of care. Presenting complaint: Patient states "I'm here for depression I don't have any energy, and I can't think or breath and my thoughts race" for approx 30 days. . ca13

09:05 Method Of Arrival: Walk In (Triage) ca13

13:58 Acuity: 3 U ca13

13:58 Acuity 3 U ca13

## Triage Assessment:

09:13 General: Appears in no apparent distress, comfortable, Behavior is cooperative, quiet. ca13

## MSE:

10:17 Financial registration completed. ac47

## Historical:

- Allergies: PENICILLINS;

- Home Meds:

1. Paxil 20 mg Oral tab 1 tab once daily

2. Hydrocodone-Acetaminophen Oral Unknown

- PMHx: No Past Medical History;

- PSHx: Hernia repair; Hip Surgery;

## Historical:

09:07 Social history Smoking status: Patient states was never smoker of tobacco. Patients PREFERRED language is English. Communicable Disease Screen: Travel: Have you or any of your close contacts traveled outside of the USA in the past three weeks? No. Exposure: Have you been exposed to someone with a suspected Ebola, MERS, tuberculosis (TB) , chicken pox/shingles or other infectious illness in the past ca13

**Brown, Baron Lloyd**

Enterprise Patient ID: 1010937371

Medical Record #: 10479960 / Account #: 62744265

ED Nurse Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 1 of 5

**BAYLOR UNIVERSITY MEDICAL CENTER**

PATIENT NAME: Brown, Baron Lloyd  
 MRN / ACCOUNT #: 10479960 / 62744265  
 DOB / AGE / SEX: 10/26/1958 / 61Y / M

ADMIT DATE: 01/31/2019 09:01 AM  
 DICTATION DATE/TIME:  
 PROVIDER: Seungdamrong, Jason

**ED NURSE NOTES**

three weeks? No. Signs & Symptoms: The patient exhibits the following signs and/or symptoms: None. TB Risk/Actions: Symptoms: Patient denies having a fever, night sweats, persistent cough, bloody sputum, and unexplained weight loss. Risk factors: None Negative TB screen. Suicide Screen: 1. Over the past two weeks, have you felt little interest or pleasure in doing things? Several days (1). 2. Over the past two weeks, have you felt down, depressed, or hopeless? Nearly every day (3). PHQ2 Score: 4. 4. Patient presents with positive suicidal gestures. No. 5. Over the past two weeks, have you had thoughts that you would be better off dead or hurting yourself in some way? Yes. Suicide Risk Assessment: 1. Have you wished you were dead or wished you could go to sleep and not wake up? No. 2. Have you had any actual thoughts of killing yourself? 2. Yes. 3. Have you been thinking about how you might do this? No. 4. Have you had these thoughts and had some intention of acting on them? No. 5. Have you started to work out or worked out the details of how to kill yourself and do you intend to carry out this plan? No. 6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? No. Patient has low suicide risk. SAFETY CHECKLIST -.

09:12 "Does the patient present with an immediate risk of danger to others?" No. ca13

10:09 Social history Patient/guardian denies using street drugs, IV drugs. Immunization History: Noncontributory. Code Status: Full Code (FC). ca13

13:45 The history from nurses notes was reviewed and I agree with what is documented. js75

**Screening:**

09:12 Abuse screen: Denies threats or abuse. Denies injuries from another. Johns Hopkins Fall Risk Assessment: Mobility - no risk identified ca13  
 (\*\*0\*\*) Cognition - no risk identified (\*\*0\*\*) Elimination - no incontinence/urgency/frequency (\*\*0\*\*) Fall history - None within 6 months (\*\*0\*\*) Medication - No high fall risk drugs (\*\*0\*\*) Age - 60 - 69 years old (\*\*1\*\*) Patient care equipment that may tether patient - One present (\*\*1\*\*). The patient's Fall Risk Score is 1 (low risk).

**Assessment:**

10:09 Nursing Diagnosis: Impaired comfort related to illness, insufficient resources, or lack of privacy and/control. Nursing Goal: Improve patient's comfort level. Nursing plan: Monitor patient's vital signs. ca13  
 General: Appears in no apparent distress, comfortable, Behavior is cooperative, quiet, Bed in low position. Call light in reach. Side rails up X 1. Adult with Patient. Neuro: Level of Consciousness is awake, obeys commands. Cardiovascular: Chest pain is denied. Respiratory: Airway is patent Respiratory effort is even, unlabored, Respiratory pattern is regular, symmetrical. Derm: Skin is warm &

**Brown, Baron Lloyd**

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ED Nurse Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 2 of 5

**BAYLOR UNIVERSITY MEDICAL CENTER**

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**ED NURSE NOTES**

dry. Psych Assessment: Objective: Affect is flat, Patient is cooperative.

10:29 Continuation note: UBH (Richard) contacted. Assessor en route and will return call for ETA. mn7

10:36 Continuation note: Sumiaya UBH assessor's ETA: Within the hour. mn7

11:35 Continuation note: Sumiaya UBH at bedside. mn7

13:57 General: Appears in no apparent distress, comfortable, Behavior is cooperative. Neuro: Level of Consciousness is awake, obeys commands. ca13  
 Respiratory: Airway is patent Respiratory effort is even, unlabored, Respiratory pattern is regular, symmetrical. Derm: Skin is warm ` dry.

Vital Signs:

09:12 BP 168 / 136 LA Sitting (auto/lg); Pulse 72; Resp 18; Temp 98.2(O); ca13  
 Pulse Ox 99% on R/A; Weight 90.72 kg (R); Height 6 ft. 3 in. (190.50 cm) (R);

10:10 BP 164 / 107 LA Sitting (auto/lg); Pulse 68; Resp 18; Pulse Ox 98% on ca13  
 R/A;

13:50 BP 164 / 110 LA Sitting (auto/lg); Pulse 62; Resp 18; Pulse Ox 97% on ca13  
 R/A;

09:12 Body Mass Index 25.00 (90.72 kg, 190.50 cm) ca13

Vitals:

09:12 Sepsis Screen: No SIRS criteria identified at this time, No SIRS ca13  
 criteria identified at this time. No suspected infection identified at this time. Patient has a negative sepsis screen.

10:09 Patient complains of pain. ca13

Glasgow Coma Score:

10:09 Eye Response: spontaneous(4). Verbal Response: oriented(5). Motor ca13  
 Response: obeys commands(6). Total: 15.

13:50 Eye Response: spontaneous(4). Verbal Response: oriented(5). Motor ca13  
 Response: obeys commands(6). Total: 15.

ED Course:

09:03 Patient arrived in ED. ac47

09:03 Patient visited by Cabrera, Annabel, Access Serv. ac47

09:03 Patient status - Bed L Hall 1 ac47

09:05 Patient status - Bed L2 mn7

09:05 Feist, Catherine, RN is Primary Nurse. ca13

09:06 Seungdamrong, Jason, MD is Attending Physician. js75

09:06 Patient visited by Seungdamrong, Jason, MD js75

09:13 Patient visited by Feist, Catherine, RN ca13

09:13 Patient has correct name band on right wrist for positive ca13  
 identification.

09:53 Patient visited by Seungdamrong, Jason, MD js75

**Brown, Baron Lloyd**

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ED Nurse Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 3 of 5

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 DICTATION DATE/TIME:  
 PROVIDER: Seungdamrong, Jason

**ED NURSE NOTES**

10:00 Patient visited by Cabrera, Annabel, Access Serv. ac47  
 10:10 Patient's medication allergies reviewed and updated in MedHost. ED ca13  
 Unit Secretary notified to update allergies in Eclipsys. NIBP on.  
 Pulse oximeter on patient. Primary support person: significant other.  
 10:10 Specimen identification verified by patient and RN prior to sending ca13  
 to lab. Blood drawn from right arm Urine collected. Sent per order to  
 lab.  
 10:17 Patient visited by Cabrera, Annabel, Access Serv. ac47  
 10:28 POC Comp Panel Reviewed. js75  
 10:40 Patient visited by Nguyen, Mai, RN mn7  
 12:03 Patient visited by Feist, Catherine, RN ca13  
 12:18 Level - Acetaminophen (Tylenol) Reviewed. js75  
 12:18 Level - Salicylate (Aspirin) Reviewed. js75  
 12:18 Level - ETOH (Alcohol) Serum Reviewed. js75  
 12:18 UDS (Urine Drug Screen) Reviewed. js75  
 13:58 Triage completed. ca13

**Administered Medications:**

No medications were administered

**Outcome:**

13:41 Disposition indicated by Provider. js75  
 13:57 Condition: Stable Discharged to home ambulatory with family. ca13  
 13:57 Nursing diagnosis outcomes for alteration in comfort: Goal met.  
 13:57 Discharge instructions given to patient, Instructed on adequate  
 hydration. discharge instructions, follow up and referral plans,  
 resources provided by UBH Verbalized understanding of instructions,  
 medications, the need for follow up, Transition of care record given  
 including diagnosis, procedures/tests performed during visit,  
 Medication Reconciliation form and any additional instructions.  
 13:57 Discharge Assistance: Patient does not request additional resources  
 at discharge.  
 13:57 Patient left the emergency department at: 13:58.  
 13:58 Patient encounter removed from the MedHost active patient list. ca13

**Signatures:**

|                                |        |      |
|--------------------------------|--------|------|
| Feist, Catherine, RN           | RN     | ca13 |
| Nguyen, Mai, RN                | RN     | mn7  |
| Seungdamrong, Jason, MD        | MD     | js75 |
| Cabrera, Annabel, Access Serv. | Access | ac47 |

**Orders:**

Order: Low Risk; Ordered: 01/31 09:11; By: ca13; For: js75;  
 Completed: 01/31 09:14; By: mn7; Order Method: Verbal - Read back;

**Brown, Baron Lloyd**

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ED Nurse Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 4 of 5



**BAYLOR UNIVERSITY MEDICAL CENTER**

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ADMIT DATE: 01/31/2019 09:01 AM  
DICTATION DATE/TIME:  
PROVIDER: Seungdamrong, Jason

**ED NURSE NOTES**

Sign Off: Seungdamrong, Jason, MD - 01/31 10:06  
Order: Level - Salicylate (Aspirin); Ordered: 01/31 09:54; By: js75;  
For: js75; Reviewed: 01/31 12:18; By: js75; Order Method:  
Electronic; Specimen: Blood; Priority: STAT  
Order: Level - Acetaminophen (Tylenol); Ordered: 01/31 09:54; By:  
js75; For: js75; Reviewed: 01/31 12:18; By: js75; Order Method:  
Electronic; Specimen: Blood; Priority: STAT  
Order: CBC (Complete Blood Count); Ordered: 01/31 09:54; By: js75;  
For: js75; Cancelled: 01/31 10:15; By: mn7; Order Method:  
Electronic; Reason for Cancellation: Order Correction; Specimen:  
Blood; Priority: STAT  
Order: CMP (Comprehensive Metabolic Panel); Ordered: 01/31 09:54; By:  
js75; For: js75; Cancelled: 01/31 10:15; By: mn7; Order Method:  
Electronic; Reason for Cancellation: Order Correction; Specimen:  
Blood; Priority: STAT  
Order: Level - ETOH (Alcohol) Serum; Ordered: 01/31 09:54; By: js75;  
For: js75; Reviewed: 01/31 12:18; By: js75; Order Method:  
Electronic; Specimen: Blood; Priority: STAT  
Order: UDS (Urine Drug Screen); Ordered: 01/31 09:54; By: js75; For:  
js75; Reviewed: 01/31 12:18; By: js75; Order Method: Electronic;  
Specimen: Urine; Specimen Type:: Urine; Priority: STAT  
Order: Psych Consult - Depression; Ordered: 01/31 09:54; By: js75;  
For: js75; Completed: 01/31 10:29; By: mn7; Order Method: Electronic  
Order: BP Recheck; Ordered: 01/31 10:06; By: js75; For: js75;  
Completed: 01/31 10:08; By: cal3; Order Method: Electronic  
Order: LL POC CBC; Ordered: 01/31 10:15; By: js75; For: js75;  
Completed: 01/31 10:15; By: mn7; Order Method: Electronic  
Order: LL POC Comp Panel; Ordered: 01/31 10:15; By: js75; For: js75;  
Completed: 01/31 10:15; By: mn7; Order Method: Electronic  
Order: GFR AA and Non AA; Ordered: 01/31 10:15; By: dispat; For:  
js75; Cancelled: 01/31 10:15; By: dispat; Sign Off: ; Reason for  
Cancellation: Test GFR AA and Non AA was cancelled on 01/31/2019 at  
10:15; RBS\_TRGC\_CANC  
Order: POC Comp Panel; Ordered: 01/31 10:25; By: dispat; For: js75;  
Reviewed: 01/31 10:28; By: js75; Order Method: Electronic

## Order Signatures:

Seungdamrong, Jason, MD, MD, js75;  
Feist, Catherine, RN, RN, cal3;  
Dispatcher, HL7, RN, RN, dispat;

\*\*\*\*\*

**Brown, Baron Lloyd**

Enterprise Patient ID: 1010937371

Medical Record #: 10479960 / Account #: 62744265

ED Nurse Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 5 of 5

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ADMIT DATE: 01/31/2019 09:01 AM  
 DICTATION DATE/TIME:  
 PROVIDER: Seungdamrong, Jason

**ED PHYSICIAN NOTES**

## Physician Documentation

Baylor University Medical Center

Name: Baron Brown

Age: 60 yrs

Sex: Male

DOB: 10/26/1958

MRN: 10479960

Arrival Date: 01/31/2019

Time: 09:01

Account#: 62744265

Bed L2

Private MD:

ED Physician Seungdamrong, Jason

Diagnosis: Depression; Adjustment disorder, unspecified

## HPI:

01/31

09:55 This 60 yrs old Black Male presents to ER via Walk In (Triage) with js75  
 complaints of Depression.

09:55 The patient presents to the emergency department with depression, js75  
 over work.

09:55 Onset: The symptoms/episode began/occurred approx 30 days. Associated js75  
 signs and symptoms: Pertinent negatives: chest pain, fever, shortness  
 of breath, vomiting. Severity of symptoms: At their worst the  
 symptoms were moderate. After initial evaluation, subsequent history  
 was obtained from as per fiancée, pt has been stressed out from work.  
 pt states he has recently been stressed out from his job, got a  
 promotion but had difficulty handling new position and was demoted.  
 pt has been having intermittent suicidal thoughts, no current plan.  
 pt also states he was at JPS approx 3 days ago, received Paxil rx.

## Historical:

- Allergies: PENICILLINS;

- Home Meds:

1. Paxil 20 mg Oral tab 1 tab once daily

2. Hydrocodone-Acetaminophen Oral Unknown

- PMHx: No Past Medical History;

- PSHx: Hernia repair; Hip Surgery;

## Historical:

09:07 Social history Smoking status: Patient states was never smoker of ca13  
 tobacco. Patients PREFERRED language is English. Communicable Disease  
 Screen: Travel: Have you or any of your close contacts traveled  
 outside of the USA in the past three weeks? No. Exposure: Have you  
 been exposed to someone with a suspected Ebola, MERS, tuberculosis  
 (TB) , chicken pox/shingles or other infectious illness in the past

**Brown, Baron Lloyd**

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ED Physician Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 1 of 4

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ADMIT DATE: 01/31/2019 09:01 AM  
 DICTATION DATE/TIME:  
 PROVIDER: Seungdamrong, Jason

**ED PHYSICIAN NOTES**

three weeks? No. Signs & Symptoms: The patient exhibits the following signs and/or symptoms: None. TB Risk/Actions: Symptoms: Patient denies having a fever, night sweats, persistent cough, bloody sputum, and unexplained weight loss. Risk factors: None Negative TB screen. Suicide Screen: 1. Over the past two weeks, have you felt little interest or pleasure in doing things? Several days (1). 2. Over the past two weeks, have you felt down, depressed, or hopeless? Nearly every day (3). PHQ2 Score: 4. 4. Patient presents with positive suicidal gestures. No. 5. Over the past two weeks, have you had thoughts that you would be better off dead or hurting yourself in some way? Yes. Suicide Risk Assessment: 1. Have you wished you were dead or wished you could go to sleep and not wake up? No. 2. Have you had any actual thoughts of killing yourself? 2. Yes. 3. Have you been thinking about how you might do this? No. 4. Have you had these thoughts and had some intention of acting on them? No. 5. Have you started to work out or worked out the details of how to kill yourself and do you intend to carry out this plan? No. 6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? No. Patient has low suicide risk. SAFETY CHECKLIST -.

09:12 "Does the patient present with an immediate risk of danger to others?" No. ca13

10:09 Social history Patient/guardian denies using street drugs, IV drugs. Immunization History: Noncontributory. Code Status: Full Code (FC). ca13

13:45 The history from nurses notes was reviewed and I agree with what is documented. js75

## ROS:

09:55 Psych: Positive for depression. Constitutional: Negative for fevers Cardiovascular: Negative for chest pain. All other systems reviewed and are negative. js75

## Exam:

09:55 The vital signs have been reviewed and are abnormal: The patient is hypertensive. js75

09:55 Psych: Behavior/mood is depressed, Affect is calm, oriented to person, place and time.

09:55 Constitutional: Awake, and alert Head/Face: Normocephalic, atraumatic. Eyes: Extra-ocular motions intact. ENT: Nares patent. No external deformities. Mucous membranes moist. Neck: Trachea midline. No external deformities. No meningismus. Cardiovascular: S1 and S2 present. No peripheral cyanosis or mottling appreciated Respiratory: No increased work of breathing, no retractions or nasal flaring. Abdomen/GI: Soft, non-tender. No guarding or rebound. No evidence of tenderness throughout. Skin: Warm and dry with good turgor. No acute lesions appreciated MS/ Extremity: No cyanosis. No

**Brown, Baron Lloyd**

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ED Physician Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 2 of 4

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 PROVIDER: Seungdamrong, Jason

**ED PHYSICIAN NOTES**

acute deformities

## Vital Signs:

09:12 BP 168 / 136 LA Sitting (auto/lg); Pulse 72; Resp 18; Temp 98.2(O); ca13  
 Pulse Ox 99% on R/A; Weight 90.72 kg (R); Height 6 ft. 3 in. (190.50  
 cm) (R);  
 10:10 BP 164 / 107 LA Sitting (auto/lg); Pulse 68; Resp 18; Pulse Ox 98% on ca13  
 R/A;  
 13:50 BP 164 / 110 LA Sitting (auto/lg); Pulse 62; Resp 18; Pulse Ox 97% on ca13  
 R/A;  
 09:12 Body Mass Index 25.00 (90.72 kg, 190.50 cm) ca13

## Glasgow Coma Score:

10:09 Eye Response: spontaneous(4). Verbal Response: oriented(5). Motor ca13  
 Response: obeys commands(6). Total: 15.  
 13:50 Eye Response: spontaneous(4). Verbal Response: oriented(5). Motor ca13  
 Response: obeys commands(6). Total: 15.

## MDM:

09:53 Patient medically screened. js75  
 13:43 Differential diagnosis: acute psychotic break, depression, drug js75  
 withdrawal. metabolic disorder, psychosis secondary to  
 non-compliance. Data reviewed: Vital signs, nurse's notes, old  
 medical records. Counseling: I had a detailed discussion with the  
 patient/guardian regarding the history, exam, and diagnostic results  
 supporting the diagnosis. I discussed a treatment plan with the  
 patient/guardian and answered questions. They agreed with the plan of  
 care. lab results, Counseling: I had a detailed discussion with the  
 patient and/or guardian regarding: the need for outpatient follow-up.  
 Response to treatment: the patient's symptoms have markedly improved  
 after treatment. Physician consultation: pt s/p behavioral health  
 evaluation who have arranged for outpatient daytime hospitalization,  
 and state no inpatient treatment needed, no current psychiatric  
 emergency requiring inpatient hospitalization.. ED course: pt states  
 no current suicidal thoughts. no HI, AVH.

01/31  
 09:54 Order name: Level - Salicylate (Aspirin); Complete Time: 12:18 js75  
 01/31  
 09:54 Order name: Level - Acetaminophen (Tylenol); Complete Time: 12:18 js75  
 01/31  
 09:54 Order name: Level - ETOH (Alcohol) Serum; Complete Time: 12:18 js75  
 01/31  
 09:54 Order name: UDS (Urine Drug Screen); Complete Time: 12:18 js75  
 01/31

**Brown, Baron Lloyd**

Enterprise Patient ID: 1010937371

Medical Record #: 10479960 / Account #: 62744265

ED Physician Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 3 of 4

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PROVIDER: Seungdamrong, Jason

**ED PHYSICIAN NOTES**

09:11 Order name: Low Risk; Complete Time: 09:14 ca13  
01/31  
09:54 Order name: Psych Consult - Depression; Complete Time: 10:29 js75  
01/31  
10:06 Order name: BP Recheck; Complete Time: 10:08 js75  
01/31  
10:15 Order name: LL POC CBC; Complete Time: 10:15 js75  
01/31  
10:15 Order name: LL POC Comp Panel; Complete Time: 10:15 js75  
01/31  
10:25 Order name: POC Comp Panel; Complete Time: 10:28 dispat

Dispensed Medications:  
No medications were administered

**Disposition:**

01/31/19 13:41 Discharged to Home. Impression: Adjustment disorder,  
unspecified, Depression.  
- Condition is Stable.  
- Discharge Instructions: Adjustment Disorder, Adult, How to Take  
Your Blood Pressure, Easy-to-Read.  
  
- Medication Reconciliation form.  
- Follow up: Private Physician; When: 1 - 2 days; Reason: Recheck  
today's complaints.  
- Problem is new.  
- Symptoms have improved.

**Signatures:**

|                         |    |       |
|-------------------------|----|-------|
| Dispatcher, HL7, RN     | RN | dispa |
| Feist, Catherine, RN    | RN | ca13  |
| Nguyen, Mai, RN         | RN | mn7   |
| Seungdamrong, Jason, MD | MD | js75  |

\*\*\*\*\*

**Brown, Baron Lloyd**

Enterprise Patient ID: 1010937371

Medical Record #: 10479960 / Account #: 62744265

ED Physician Notes

TRANSCRIPTION DATE/TIME: 2/1/2019 7:58 AM

Page 4 of 4

Patient: BARON BROWN  
Medical Record: #107145

HOPE - ADMISSION

Section A: Administrative Information

Patient / Admission Information

A0215. Site of service at admission 01. Patient's Home/Residence  
A0220. Admission date 12/30/2025  
A0500A. Patient first name BARON  
A0500B. Patient middle initial L  
A0500C. Patient last name BROWN  
A0550. Patient zip code 76112-0001  
A0600A. Social Security Number 453-21-8528  
A0600B. Patient Medicare/railroad insurance number 9W00K37ER02  
☒ N. Patient is not a Medicaid recipient  
A0810. Sex 1. Male  
A0900. Birthdate 10/26/1958

A1005. Ethnicity

☒ A1005A. No, not of Hispanic, Latino/a, or Spanish origin

A1010. Race

☒ A1010B. Black or African American

Language

A1110A. Preferred Language English  
A1110B. Interpreter requested 0. No

Payor Information

☒ A1400A. Medicare (FFS)

Admitted From

A1805. Admitted From 01. Home/Community (pvt home/apt, board/care, alf, etc)

Living Arrangements

A1905 Living Arrangements 2. With others in the home (family, friends, paid caregiver)

Availability of Assistance

A1910 Availability of Assistance 1. Around-the-clock (24 hours a day with few exceptions)

Section F: Preferences

Was patient/responsible party asked about CPR 1. Yes  
F2000B. Date asked about CPR 12/30/2025  
Was patient/responsible party asked about treatements other than CPR 1. Yes  
F2100B. Date asked about treatment other than CPR 12/30/2025  
Was patient/responsible party asked about hospitalization 1. Yes  
F2200B. Date asked about hospitalization 12/30/2025  
Was patient/responsible party asked about spiritual/existential concerns 1. Yes  
F3000B. Date asked about spiritual/existential concerns 12/30/2025

Section I: Active Diagnoses

I0010. Principal diagnosis 05. Chronic Obstructive Pulmonary Disease (COPD)

Comorbidities and Co-existing Conditions

- ☒ I0600. Heart Failure (e.g., congestive heart failure (CHF) and pulmonary edema)
- ☒ I0900. Peripheral Vascular Disease (PVD) or Peripheral Arterial Disease (PAD)
- ☒ I1510. Renal disease

Section J: Health Conditions

Pain

- J0900A. Was patient screened for pain1. Yes
- J0900B. Date of first screening for pain12/30/2025
- J0900C. Patient's pain severity was2. Moderate
- J0900D. Type of standardized pain tool used1. Numeric
- J0905. Is pain an active problem for the patient?1. Yes
- J0910A. Was comprehensive pain assessment done1. Yes
- J0910B. Date of comprehensive pain assessment12/30/2025

Comprehensive pain assessment included:

- ☒ J0910C1. Location
- ☒ J0910C2. Severity
- ☒ J0910C3. Character
- ☒ J0910C4. Duration
- ☒ J0910C5. Frequency
- ☒ J0910C6. What relieves/worsens pain
- ☒ J0910C7. Effect on function or quality of life

J0915. Neuropathic Pain1. Yes

Respiratory Status

- J2030A. Was patient screened for shortness of breath1. Yes
- J2030B. Date of first screening for shortness of breath12/30/2025
- J2030C. Did screening indicate the patient had shortness of breath1. Yes
- J2040A. Was treatment for shortness of breath initiated2. Yes
- J2040B. Date treatment for shortness of breath initiated12/30/2025

Symptom Impact

- J2050A. Was a symptom impact screening completed?1. Yes
- J2050B. Date of symptom impact screening12/30/2025
- J2051/J2053. Symptom Impact

|                        | J2051 Symptom Impact Screening | J2053 Symptom Impact Screening |
|------------------------|--------------------------------|--------------------------------|
| A. Pain                | 1. Slight                      |                                |
| B. Shortness of breath | 1. Slight                      |                                |
| C. Anxiety             | 1. Slight                      |                                |
| D. Nausea              | 0. Not at all                  |                                |
| E. Vomiting            | 0. Not at all                  |                                |
| F. Diarrhea            | 0. Not at all                  |                                |
| G. Constipation        | 1. Slight                      |                                |
| H. Agitation           | 0. Not at all                  |                                |

Section M: Skin Conditions

**M1190 Does the patient have one or more skin conditions?** 0. No

**Section N: Medications**

---

- N0500A. Was scheduled opioid initiated or continued

0. No
- N0510A. Was PRN opioid initiated or continued

1. Yes
- N0510B. Date PRN opioid initiated or continued

12/30/2025
- N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")

2. Yes
- N0520B. Date bowel regimen initiated or continued

12/30/2025

**Section Z: Record Administration**

---

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

12/31/2025

**Electronically signed by:** VIRGINIA ALLAN RN ADON



Patient: **BARON BROWN**  
Medical Record: **#107145**

COMFORT KIT ORDERS

All of the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring

- ☒ Morphine Sulfate, 100mg/5ml = 20 mg/ml, 0.25-1ml, Sublingual, Q1-2 hours prn, Pain/SOB - #120mL, 00054040441
- ☒ Lorazepam Liquid, 1.75mg/ml, 0.5-1 mg, PO/SL, q4 hrs prn, Anxiety/SOB - #30mL / RF: 5, 00054353244
- ☒ Atropine sulfate oral Soluntion, 0.75%, 1-2 drops, Take Sublingually, every 2 hours as needed, for excess secretions - #15mL / RF: 11
- ☒ APAP 650MG SUPPS, 650 mg, 1 suppository, unwrap and insert Rectally, Q 4 HRS PRN, Pain/Fever - #4 / RF:11 , 00927067812
- ☒ Bisacodyl suppository, 10 mg, 1 suppository, unwrap and insert Rectally, Q DAY PRN, Constipation - #3 / RF:11 , 00363010908
- ☒ Zofran ODT, 1 tablet, 4mg, dissolve PO/SL, Q 8 HRS PRN, nausea/vomiting - #6 / RF:11
- ☒ Senna, 8.6mg, 1 Tab, PO, Daily, Constipation
- ☒ Oxygen, na, 2-4 liters, intranasally, PRN, Shortness of breath

Ordering physician: DIEGO F RESTREPO MD  
Date: 12/30/2025

Electronically signed by: TRACY KING RN  
Electronically signed by: DIEGO F RESTREPO MD

## Revocation of Election to Hospice Care

Homage Hospice

Patient's Name: Baron Brown ID# \_\_\_\_\_

Medicare Number \_\_\_\_\_ Effective Date of Revocation \_\_\_\_\_

### Revocation Statement

I, Baron Brown revoke my election for Medicare coverage of hospice care for the remainder of this benefit period \_\_\_\_\_.

I understand that I will lose the remaining days of this benefit period.

I understand I now resume Medicare coverage of the benefits waived when I elected the Hospice Benefit.

I understand that I may, at any time, elect to receive hospice coverage for other hospice benefit periods that I am eligible to receive. I will be electing benefit period \_\_\_\_\_.  
(Number)

The reason for revocation is \_\_\_\_\_  
\_\_\_\_\_  
\_\_\_\_\_

x Baron H. Brown \_\_\_\_\_

Patient's Signature

Date

Hospice's Staff Signature

Date

Patient: **BARON BROWN**  
Medical Record: **#107145**

# COVID-19 ASSESSMENT

## COVID-19 ASSESSMENT

---

This assessment tool supports you in identifying patients at risk for COVID-19 and helps guide clinicians in clinical decision making.

### Symptoms

---

*Note: Symptoms may appear 2-14 days after exposure.*

☒ **None**

### Travel History

---

*\*\*Updated information on affected countries can be found at: <https://www.cdc.gov/coronavirus/2019-ncov/travelers/index.html>*

### Action Taken

---

☒ **None indicated**

*\*\*Please refer to the CDC COVID-19 website for ongoing updates and guidance: <https://www.cdc.gov/coronavirus/2019-ncov>*

## COVID-19 TESTING

---

**Patient testing completed?** No

---

**Date:** 12/30/2025

**Electronically signed by:** TRACY KING RN

Patient: **BARON BROWN**  
Medical Record: **#107145**

# LCD - CARDIAC

## LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

### Worsening of Clinical Status

- ☒ Recurrent or intractable infection; pneumonia, sepsis, UTI
- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss
- ☒ Dysphagia leading to recurrent aspiration and/or decreased oral intake documented by decreasing food portion consumption

### Worsening of Symptoms

- ☒ Dyspnea with increasing respiratory rate
- ☒ Nausea, vomiting poorly responsive to treatment
- ☒ Intractable diarrhea

### Worsening of Signs

- ☒ Edema
- ☒ Weakness
- ☒ Change in level of consciousness
  
- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs
- ☒ Increased ER visits, hospitalizations, or physician visits related to the hospice primary diagnosis

## Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ Ischemic heart disease
- ☒ Dementia

## Cardiac

**NEW YORK HEART ASSOCIATION CLASS** (Heart disease)

- ☒ Class III (Moderate) Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes fatigue, palpitation, or dyspnea.

### HEART DISEASE

Patients should meet criteria in numbers 1 and 2. Supporting documentation is found in no. 3.

- ☒ 1: Patient has already been optimally treated with vasodilators unless contraindicated AND is not a candidate for surgery or has refused surgery

3: Any of the following will help support the diagnosis but is not required:

- ☒ **Treatment resistant symptomatic supraventricular or ventricular arrhythmias.**
- ☒ **History of unexplained syncope**

**Date:** 12/30/2025 9:08:00 AM

**Electronically signed by:** TRACY KING RN

Patient: **BARON BROWN**  
Medical Record: **#107145**

# LCD - GENERAL DECLINE IN CLINICAL STATUS

## LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

### Worsening of Clinical Status

- ☒ **Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss**

### Worsening of Signs

- ☒ **Edema**
- ☒ **Weakness**
  
- ☒ **Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease**
- ☒ **Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)**
- ☒ **Progressing dependence for performance of ADLs**

## Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ **CHF**
- ☒ **Diabetes Mellitus**
- ☒ **Dementia**

**Date:** 12/30/2025 9:09:00 AM

**Electronically signed by:** TRACY KING RN

Patient: **BARON BROWN**  
Medical Record: **#107145**

# LCD - CARDIAC

## LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

### Worsening of Clinical Status

- ☒ Recurrent or intractable infection; pneumonia, sepsis, UTI
- ☒ Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss
- ☒ Dysphagia leading to recurrent aspiration and/or decreased oral intake documented by decreasing food portion consumption

### Worsening of Symptoms

- ☒ Dyspnea with increasing respiratory rate
- ☒ Nausea, vomiting poorly responsive to treatment
- ☒ Intractable diarrhea

### Worsening of Signs

- ☒ Edema
- ☒ Weakness
- ☒ Change in level of consciousness
  
- ☒ Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease
- ☒ Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)
- ☒ Progressing dependence for performance of ADLs
- ☒ Increased ER visits, hospitalizations, or physician visits related to the hospice primary diagnosis

## Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ Ischemic heart disease
- ☒ Dementia

## Cardiac

### NEW YORK HEART ASSOCIATION CLASS (Heart disease)

- ☒ Class III (Moderate) Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes fatigue, palpitation, or dyspnea.

### HEART DISEASE

Patients should meet criteria in numbers 1 and 2. Supporting documentation is found in no. 3.

- ☒ 1: Patient has already been optimally treated with vasodilators unless contraindicated AND is not a candidate for surgery or has refused surgery

3: Any of the following will help support the diagnosis but is not required:

- ☒ **Treatment resistant symptomatic supraventricular or ventricular arrhythmias.**
- ☒ **History of unexplained syncope**

---

**Date:** 12/30/2025 12:24:00 PM

**Electronically signed by:** TRACY KING RN



Patient: **BARON BROWN**  
Medical Record: **#107145**

# LCD - GENERAL DECLINE IN CLINICAL STATUS

## LCD Factors

These are the LCD guidelines to consider when determining a prognosis of six months or less.

### Worsening of Clinical Status

- ☒ **Weight loss not due to reversible causes. If you can't weigh the patient, recumbent measurements (mid-arm circumference, abdominal girth) can be used to document weight loss**

### Worsening of Signs

- ☒ **Edema**
- ☒ **Weakness**
  
- ☒ **Decline in Karnofsky Performance Scale (KPS) or Palliative Performance Scale (PPS) from <70% due to progression of disease**
- ☒ **Decline in Functional Assessment (FAST) for dementia (from =7A on the FAST)**
- ☒ **Progressing dependence for performance of ADLs**

## Comorbidities

The presence of comorbidities may hasten the patient's clinical progression and should be identified and addressed.

- ☒ **CHF**
- ☒ **Diabetes Mellitus**
- ☒ **Dementia**

**Date:** 12/30/2025 12:25:00 PM

**Electronically signed by:** TRACY KING RN

Patient: **BARON BROWN**  
Medical Record: **#107145**

# NURSE PROGRESS NOTE

NURSE PROGRESS NOTE

**Date/Time in:** 1/14/2026 4:01:00 PM  
**Date/Time out:** 1/14/2026 4:02:00 PM  
**Purpose of progress note:** IDG Narrative  
**IDG Narrative** 67-year-old male on service for heart disease. Symptoms well managed POC is effective.

**Date:** 1/14/2026

**Electronically signed by:** BETTY NYINDODO RN

Patient: **BARON BROWN**  
Medical Record: **#107145**

# ADMISSION ORDERS / INITIAL PLAN OF CARE

## Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.

## Nutrition Orders

- ☒ Oral diet orders
- Describe: Diet as tolerated

## Activity Orders

- ☒ As tolerated

## Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

## INITIAL PLAN OF CARE

### Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 1
- +PRN Weekly Visits 1
- Reason for PRN Visits Change in condition

### Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment

- ☒ Assist patient/family in planning future care
- ☒ Assist with access to community resources
- ☒ Assist with Advance Directives
- ☒ Assist with funeral planning
- ☒ Assist patient/family in obtaining financial assistance
- ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
- ☒ Evaluation for volunteer services

Visit Frequency for next month (to be reassessed at next IDG):1

+PRN Visits for next month:1

Spiritual Visits

- ☒ Spiritual needs assessment
- ☒ Phone contacts and/or visits to patient/family
- ☒ Phone call to patient/family's own pastor
- ☒ Patient/family receive visits from their own pastor

Visit Frequency for next month (to be reassessed at next IDG): 1

+PRN Visits for next month:1

Hospice Aide Visits

- ☒ Patient/family request no Hospice Aide visits at this time

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Clinical Director:TRACY KING RN

Registered Nurse:ANA ENCINAS RN

Social Worker:ASHLEY DAVIS LMSW

Spiritual Counselor:BRETT BRATCHER M.DIV

Volunteer Coordinator:VERNON MORRISON

Bereavement Coordinator:VERNON MORRISON

Date: 12/30/2025

Electronically signed by: VERNON MORRISON  
Electronically signed by: ASHLEY DAVIS LMSW  
Electronically signed by: ANA ENCINAS RN  
Electronically signed by: BRETT BRATCHER M.DIV

**Electronically signed by:** TRACY KING RN  
**Electronically signed by:** DIEGO F RESTREPO MD

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Anxiety/Agitation  
**Measurable Goals:** Patient and family/caregiver will report understanding of the causes and treatment of anxiety.

| Update # | Problem description | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                    | Date                  | Signature        |
|----------|---------------------|----------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|------------------|
| 1        | Anxiety/Agitation   | 1              | Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind | 12/30/2025<br>9:11 AM | TRACY<br>KING RN |
| 2        | Anxiety/Agitation   | 1              | Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind | 1/14/2026<br>9:15 PM  | TRACY<br>KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Confusion  
**Measurable Goals:** Patient and family will report understanding of the patient's confusion.

| Update # | Problem description     | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                               | Date               | Signature     |
|----------|-------------------------|----------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|---------------|
| 1        | potential for Confusion | 1              | Assess for possible causes of acute confusion: Electrolyte imbalance, Infection, Fecal impaction, Urinary retention, Side effects of medication, Pain, Hypoxia, Heart or liver failure, Alcohol withdrawal, Brain metastasis, Fear of death or pain, Sleep deprivation, Initiation of opioid therapy | 12/30/2025 9:13 AM | TRACY KING RN |
| 2        | potential for Confusion | 1              | Assess for possible causes of acute confusion: Electrolyte imbalance, Infection, Fecal impaction, Urinary retention, Side effects of medication, Pain, Hypoxia, Heart or liver failure, Alcohol withdrawal, Brain metastasis, Fear of death or pain, Sleep deprivation, Initiation of opioid therapy | 1/14/2026 9:13 PM  | TRACY KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Decline in Activities of Daily Living  
**Measurable Goals:** Support resident who has declined in order to lessen the likelihood of complications (i.e., pressure ulcers and contractures)

| Update # | Problem description                   | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     | Date                  | Signature     |
|----------|---------------------------------------|----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|---------------|
| 1        | Decline in Activities of Daily Living | 1              | Provide a positive atmosphere, while acknowledging the difficulty of the situation.<br>Protect patient from injury by assisting with activities and providing supportive devices as needed.<br>Provide patient education handout(s): Bathing a Loved One, Changing Sheets for a Bedridden Patient, Moving Someone in Bed, Hospital Beds - General Safety Guidelines, Using Medical Equipment at Home Safely, Complications of Immobility, Getting the Physically Dependent Person Off the Floor, Ways of Assisting Mobility, Dementia, Stroke, Becoming a Primary Caregiver, When You Need More Help, Your Friend Needs Help, Body Mechanics, Falls, Home Safety Checklist | 12/30/2025<br>9:15 AM | TRACY KING RN |
| 2        | Decline in Activities of Daily Living | 1              | Provide a positive atmosphere, while acknowledging the difficulty of the situation.<br>Protect patient from injury by assisting with activities and providing supportive devices as needed.<br>Provide patient education handout(s): Bathing a Loved One, Changing Sheets for a Bedridden Patient, Moving Someone in Bed, Hospital Beds - General Safety Guidelines, Using Medical Equipment at Home Safely, Complications of Immobility, Getting the Physically Dependent Person Off the Floor, Ways of Assisting Mobility, Dementia, Stroke, Becoming a Primary Caregiver, When You Need More Help, Your Friend Needs Help, Body Mechanics, Falls, Home Safety Checklist | 1/14/2026<br>9:12 PM  | TRACY KING RN |



Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Diarrhea  
**Measurable Goals:** Patient and family will report understanding of disease process and management of loose, frequent stools.

| Update # | Problem description    | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                           | Date                  | Signature     |
|----------|------------------------|----------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|---------------|
| 1        | potential for Diarrhea | 1              | Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other<br>Instruct patient to eat small, frequent, relaxed meals. | 12/30/2025<br>9:17 AM | TRACY KING RN |
| 2        | potential for Diarrhea | 1              | Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other<br>Instruct patient to eat small, frequent, relaxed meals. | 1/14/2026<br>9:11 PM  | TRACY KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Edema/Lymphedema  
**Measurable Goals:** Edema will be reduced or maintained, and complications of edema will be prevented or minimized.

| Update # | Problem description | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                                                                                                    | Date                  | Signature        |
|----------|---------------------|----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|------------------|
| 1        | Edema/Lymphedema    | 1              | Assess for possible causes of dependent edema:<br>Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness<br>Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity | 12/30/2025<br>9:18 AM | TRACY<br>KING RN |
| 2        | Edema/Lymphedema    | 1              | Assess for possible causes of dependent edema:<br>Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness<br>Instruct patient and family/caregiver on treatment for dependent edema: Avoid long periods of dependent positioning, Support of the affected extremities, Maintaining mobility, Maintaining skin integrity | 1/14/2026<br>9:10 PM  | TRACY<br>KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

Problem being care planned: Nausea/Vomiting  
Measurable Goals: Vomiting will be relieved or reduced.

| Update # | Problem description | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                                                                            | Date                  | Signature        |
|----------|---------------------|----------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|------------------|
| 1        | Nausea/Vomiting     | 1              | Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo<br>For N/V caused by abdominal distention, obstruction, or constipation: Facilitate bowel movements, Metoclopramide (Reglan) | 12/30/2025<br>9:19 AM | TRACY<br>KING RN |
| 2        | Nausea/Vomiting     | 1              | Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo<br>For N/V caused by abdominal distention, obstruction, or constipation: Facilitate bowel movements, Metoclopramide (Reglan) | 1/14/2026<br>9:09 PM  | TRACY<br>KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Dyspnea  
**Measurable Goals:** Patient will report decreased feelings of breathlessness and increased comfort.

| Update # | Problem description   | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                  | Date                  | Signature        |
|----------|-----------------------|----------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|------------------|
| 1        | potential for Dyspnea | 1              | Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites | 12/30/2025<br>9:25 AM | TRACY<br>KING RN |
| 2        | potential for Dyspnea | 1              | Assess for possible causes of dyspnea: Sudden onset, Arrhythmia, Pulmonary embolus,- Myocardial infarction- Hyperventilation- Anxiety, panic- Pneumothorax , Onset over hours to days:- Respiratory infection- Pleural or pericardial effusion, Onset over weeks:- Tumor growth- Anemia - Decreased metabolism- Ascites | 1/14/2026<br>9:06 PM  | TRACY<br>KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Pain  
**Measurable Goals:** Patient will report that pain is reduced to the level personally desired: \_\_\_\_\_ on a scale of 1-10

| Update # | Problem description | Severity score | Interventions/outcomes                                                                                                                                                                                                                                                                                                                                  | Date                  | Signature     |
|----------|---------------------|----------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------|---------------|
| 1        | potential for pain  | 1              | Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. | 12/31/2025<br>9:23 AM | TRACY KING RN |
| 2        | potential for pain  | 1              | Follow the WHO analgesic ladder to initiate pharmacologic interventions: Mild pain - non-opioids and adjuvants, Moderate pain - weak opioids, non-opioids, and adjuvants, Severe pain - strong opioids, non-opioids, and adjuvants, Adjuvant therapy may include: anti-depressants, anti-convulsants, steroids, anti-spasmodics, muscle relaxants, etc. | 1/14/2026<br>9:07 PM  | TRACY KING RN |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Anticipatory Grief  
**Measurable Goals:** Patient and family will express and share grief and concerns as needed in ways meaningful to them.

| Update # | Problem description            | Severity score | Interventions/outcomes                                | Date              | Signature            |
|----------|--------------------------------|----------------|-------------------------------------------------------|-------------------|----------------------|
| 1        | No DNR or funeral arrangements | 1              | Assist with funeral and/or memorial service planning. | 1/2/2026 1:57 PM  | ASHLEY DAVIS<br>LMSW |
| 2        | No DNR or funeral arrangements | 1              | Assist with funeral and/or memorial service planning  | 1/14/2026 9:04 PM | TRACY KING RN        |

Patient: **BARON BROWN**  
Medical Record: **#107145**

CARE PLAN

**Problem being care planned:** Spiritual Needs  
**Measurable Goals:** Patient and family will continue religious and spiritual practices.

| Update # | Problem description                                                                                                                                                                                                                                                                                                                                                                               | Severity score | Interventions/outcomes                                                                                                                      | Date                 | Signature               |
|----------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------|---------------------------------------------------------------------------------------------------------------------------------------------|----------------------|-------------------------|
| 1        | Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer. | 1              | Explore areas of life that have provided spiritual comfort in the past.<br>Pray with patient and/or family as requested and as appropriate. | 1/2/2026<br>8:05 PM  | BRETT BRATCHER<br>M.DIV |
| 2        | Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer. | 1              | Explore areas of life that have provided spiritual comfort in the past.<br>Pray with patient and/or family as requested and as appropriate. | 1/14/2026<br>9:05 PM | TRACY KING RN           |

Patient: **BARON BROWN**  
Medical Record: **#107145**

# CHANGE OF FREQUENCY ORDER

Frequencies are being adjusted for the following disciplines:

- ☒ **Nursing**  
Weekly Visit Frequency: 1  
+PRN Weekly Visits 1  
Reason for change: No Change
- ☒ **Social Work**  
Visit Frequency in next month: 1  
+PRN Visits in next month 1  
Reason for change: No Change
- ☒ **Spiritual**  
Visit Frequency in next month: 0  
+PRN Visits in next month 0  
Reason for change: Pt doesn't want Chaplin Services

Medical Director/Physician Designee: DIEGO F RESTREPO MD  
RN Case Manager: BETTY NYINDODO RN

Date: 1/3/2026

Electronically signed by: TRACY KING RN  
Electronically signed by: DIEGO F RESTREPO MD



Patient: **BARON BROWN**  
Medical Record: **#107145**

IDG UPDATE

From date: 1/15/2026  
To date: 1/29/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1  
+PRN Weekly Visits 1  
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1  
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 0  
+PRN Visits in next month 0

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update:

All equipment was provided at admission.

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.  
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

| Current Medications: | Start Date               | DC Date | Medication                      | Unit Strength | Dose          | Route                      | Frequency               | Indication                             | Covered By Hospice | Status |
|----------------------|--------------------------|---------|---------------------------------|---------------|---------------|----------------------------|-------------------------|----------------------------------------|--------------------|--------|
|                      | Regular Medications:     |         |                                 |               |               |                            |                         |                                        |                    |        |
|                      | No medications           |         |                                 |               |               |                            |                         |                                        |                    |        |
|                      | Comfort Kit Medications: |         |                                 |               |               |                            |                         |                                        |                    |        |
|                      | 12/30/2025               |         | APAP 650MG SUPPS                | 650 mg        | 1 suppository | unwrap and insert Rectally | Q 4 HRS PRN             | Pain/Fever - #4 / RF:11                | Yes                | Active |
|                      | 12/30/2025               |         | Atropine sulfate oral Soluntion | 0.75%         | 1-2 drops     | Take Sublingually          | every 2 hours as needed | for excess secretions - #15mL / RF: 11 | Yes                | Active |
|                      | 12/30/2025               |         | Bisacodyl suppository           | 10 mg         | 1 suppository | unwrap and insert Rectally | Q DAY PRN               | Constipation - #3 / RF:11              | Yes                | Active |

|            |  |                  |                      |            |                |                |                              |     |        |
|------------|--|------------------|----------------------|------------|----------------|----------------|------------------------------|-----|--------|
| 12/30/2025 |  | Lorazepam Liquid | 1.75mg/ml            | 0.5-1 mg   | PO/SL          | q4 hrs prn     | Anxiety/SOB - #30mL / RF: 5  | Yes | Active |
| 12/30/2025 |  | Morphine Sulfate | 100mg/5ml = 20 mg/ml | 0.25-1ml   | Sublingual     | Q1-2 hours prn | Pain/SOB - #120mL            | Yes | Active |
| 12/30/2025 |  | Oxygen           | na                   | 2-4 liters | intranasally   | PRN            | Shortness of breath          | Yes | Active |
| 12/30/2025 |  | Senna            | 8.6mg                | 1 Tab      | PO             | Daily          | Constipation                 | Yes | Active |
| 12/30/2025 |  | Zofran ODT       | 1 tablet             | 4mg        | dissolve PO/SL | Q 8 HRS PRN    | nausea/vomiting - #6 / RF:11 | Yes | Active |

Medication list reflects medication orders as of 1/15/2026

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

| Problem                               | Start Date | Latest Update |
|---------------------------------------|------------|---------------|
| Anxiety/Agitation                     | 12/30/2025 | 12/30/2025    |
| Confusion                             | 12/30/2025 | 12/30/2025    |
| Decline in Activities of Daily Living | 12/30/2025 | 12/30/2025    |
| Diarrhea                              | 12/30/2025 | 12/30/2025    |
| Edema/Lymphedema                      | 12/30/2025 | 12/30/2025    |
| Nausea/Vomiting                       | 12/30/2025 | 12/30/2025    |
| Dyspnea                               | 12/30/2025 | 12/30/2025    |
| Pain                                  | 12/31/2025 | 12/31/2025    |
| Anticipatory Grief                    | 1/2/2026   | 1/2/2026      |
| Spiritual Needs                       | 1/2/2026   | 1/2/2026      |

Care Plan list reflects care plan orders as of 1/15/2026

Treatment Order List:

| Name                | Start Date | DC Date |
|---------------------|------------|---------|
| No Physician Orders |            |         |

Physician orders list reflects physician orders as of 1/15/2026

**IDG Narrative:** Provide spiritual support through socialization, scripture and prayer 67-year-old male on service for heart disease. Symptoms well managed POC is effective.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

**Medical Director/Physician Designee:** DIEGO F RESTREPO MD  
**Registered Nurse:** BETTY NYINDODO RN  
**Social Worker:** ASHLEY DAVIS LMSW  
**Spiritual Counselor** BRETT BRATCHER M.DIV  
**Volunteer Coordinator:** VERNON MORRISON  
**Bereavement Coordinator:** VERNON MORRISON

**Date:** 1/14/2026

**Electronically signed by:** ASHLEY DAVIS LMSW  
**Electronically signed by:** VERNON MORRISON  
**Electronically signed by:** VIRGINIA ALLAN RN ADON  
**Electronically signed by:** BRETT BRATCHER M.DIV

Patient: **BARON BROWN**  
Medical Record: **#107145**

# SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

|                           |                                                                                                                                                                                                                                                       |
|---------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Date/Time in:             | 12/31/2025 3:41:00 PM                                                                                                                                                                                                                                 |
| Date/Time out:            | 12/31/2025 3:51:00 PM                                                                                                                                                                                                                                 |
| Purpose of progress note: | Phone Call(s) - Administrative                                                                                                                                                                                                                        |
| Narrative:                | After performing bereavement assessment, with patient daughter called patient who's voice mail was not activated per the automated message. Followed up with a text message to make introduction and schedule a time to perform spiritual assessment. |

Date: 12/31/2025

Electronically signed by: BRETT BRATCHER M.DIV

Patient: **BARON BROWN**  
Medical Record: **#107145**

# SPIRITUAL VISIT NOTE

## SPIRITUAL VISIT NOTE

**Date/Time in:** 1/2/2026 9:31:00 AM  
**Date/Time out:** 1/2/2026 9:51:00 AM  
**Modality:** Telehealth visit  
**COVID-19 screening performed?** No  
**Reason:** No symptoms  
**Others present during the visit:** patient  
**Patient/family/caregiver's primary concern during this visit:** no concerns expressed

## Pain Assessment

**Pain rating:** 0  
**Pain scale used:** Verbal Scale

## Current Mental Status

- ☒ Alert
- ☒ Oriented

## Current Emotional Status

- Expressive
- ☒ Patient

## Current Behavioral Status

- Appropriate
- ☒ Patient
- Cooperative
- ☒ Patient

**Describe patient's/responsible party's mood and/or affect during visit:** patient receptive to spiritual assessment and visit. Patient in good mood

**Describe patient's/responsible party's anticipatory grief status:** Neither patient nor his daughter expressed anticipatory grief

**Describe religious rituals, sacred readings, prayer, etc:** Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer.

**Linkage with referrals:** None needed:

## Teaching Provided During Visit

**Topic(s) discussed (includes topic(s) and person(s))** Educated patient on the role of the chaplain, support and resources available to him and the family. Patient affirms understanding verbally and is accepting of future chaplain visits for

receiving instruction): socialization, scripture and prayer.

• Method(s)

☒ Verbal instructions

• Outcome(s)

☒ Verbalized understanding

Communication

☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Plan for next visit: Provide spiritual support through socialization, scripture and prayer

Additional Information/Comments: Patient is a 67-year-old male residing at a private residence with a primary diagnosis of I25.10 - Athscl heart disease of native coronary artery w/o ang pctrs. Patient was away from home and unavailable to meet face to face. Patient was open to telehealth visit and was amenable to conversation about previous work history in construction, as well as discussing his hobbies of drawing, fishing, walks and community support. Patient identifies as Baptist but does not attend church consistently. Patient is in a program of recovery and routinely attends narcotics anonymous as a part of his spiritual and social support. Patient helps others recover and in turn benefits from social and emotional support from a community of recovery. Patient is open to chaplain visits for socialization, scripture and prayer. In accordance with his Christian faith and recovery background, provided the patient with active listening, affirmation of faith, emotional support and prayer. Patient expressed gratitude for the call. Will continue the current plan of care with chaplain visits 1X a month for spiritual and emotional support.

Date: 1/2/2026

Electronically signed by: BRETT BRATCHER M.DIV

Patient: BARON BROWN  
Medical Record: #107145

# SPIRITUAL PROGRESS NOTE

## SPIRITUAL PROGRESS NOTE

Date/Time in:1/2/2026 9:52:00 AM

Date/Time out:1/2/2026 9:53:00 AM

Purpose of progress note:IDG Narrative

IDG Narrative Provide spiritual support through socialization, scripture and prayer

Date: 1/2/2026

Electronically signed by: BRETT BRATCHER M.DIV

Patient: **BARON BROWN**  
Medical Record: **#107145**

# SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

**Date/Time in:** 1/11/2026 8:11:00 AM  
**Date/Time out:** 1/11/2026 8:12:00 AM  
**Purpose of progress note:** IDG Narrative  
**IDG Narrative** Provide spiritual support through socialization, scripture and prayer

**Date:** 1/11/2026

**Electronically signed by:** BRETT BRATCHER M.DIV